Your Personal Surgery Preparation Guide

Built on evidence-based clinical guidelines

Welcome! This guide was built to help you walk into your procedure prepared, confident, and genuinely ready. Take your time with it. Come back to it. This is yours.

Written for you by Dr. Ornella Oluwole
Board-Certified Anesthesiologist • OpWell Concierge™

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1

Medication & Supplement Review

What to stop, what to keep, and when

One of the most important steps you can take before surgery is reviewing every medication, supplement, and vitamin you take with your surgical team. Some common over-the-counter products can increase bleeding, interfere with anesthesia, or interact with post-surgical medications. This module walks you through exactly what to look for.

Dr. Oluwole's Tip

I always tell my patients: bring every single bottle — prescription, supplement, vitamin, herbal tea — to your pre-op appointment. You'd be surprised how many "harmless" supplements can cause serious bleeding during surgery.

Blood Thinners & NSAIDs

These medications affect how your blood clots. Your surgeon and anesthesiologist will give you specific stop dates, but here are general guidelines:

Medication	Typical Stop Time	Notes
Aspirin (low-dose 81mg)	7 days before	Unless cardiologist says continue
Ibuprofen (Advil, Motrin)	7 days before	Switch to Tylenol for pain
Naproxen (Aleve)	7–14 days before	Longer-acting NSAID — discuss with your surgeon
Eliquis (apixaban)	2–3 days before	Per prescribing physician
Xarelto (rivaroxaban)	2–3 days before	Per prescribing physician
Warfarin (Coumadin)	5 days before	May need bridging therapy

Plavix (clopidogrel)	5–7 days before	Do NOT stop without cardiologist approval

IMPORTANT

Never stop a blood thinner on your own. Always confirm with the prescribing physician — especially if you have a heart stent, atrial fibrillation, or history of blood clots.

GLP-1 Medications (Ozempic, Wegovy, Mounjaro)

If you take a GLP-1 receptor agonist for weight loss or diabetes, this is critically important. These medications slow gastric emptying, which means food can remain in your stomach much longer than normal. This creates a real aspiration risk (food or liquid entering your lungs) under anesthesia.

Current ASA (American Society of Anesthesiologists) guidance:

- **Weekly injections** (semaglutide/Ozempic, Wegovy): Skip the dose due in the **week before** surgery (effectively a 7-day hold)
- **Daily injections** (liraglutide/Saxenda): Hold the **day of surgery**
- **Tirzepatide** (Mounjaro/Zepbound): Hold for **7 days** before surgery
- If GI symptoms (nausea, vomiting, bloating) are present, consider a **longer hold** and discuss with your anesthesiologist
- Your anesthesiologist may recommend a **gastric ultrasound** on the day of surgery to check for residual stomach contents

FORGOT TO HOLD YOUR GLP-1?

If you forgot to hold your GLP-1 medication, tell your anesthesiologist immediately. Surgery may still proceed if you have no GI symptoms, but your team needs to know so they can take extra precautions.

Dr. Oluwole's Tip

I am seeing more and more patients on GLP-1 medications. If you're on one, please don't feel embarrassed to tell us — we just need to know so we can keep you safe. A full stomach under anesthesia is one of the most dangerous situations we face.

Supplements & Herbals to Stop

The general rule: **stop all supplements and herbal products at least 7–14 days before surgery** unless your doctor tells you otherwise.

Supplement	Why Stop?	Stop Timing
Fish oil / Omega-3	Increases bleeding	7–14 days
Vitamin E (high dose)	Increases bleeding	7–14 days
Garlic supplements	Increases bleeding	7 days
Ginkgo biloba	Increases bleeding	7 days
Ginseng	Affects blood sugar & bleeding	7 days
Turmeric / Curcumin	Blood-thinning properties	7–14 days
St. John's Wort	Drug interactions with anesthesia	14 days
Kava	Liver toxicity, sedation interaction	14 days
Valerian root	Sedation interaction	14 days
Echinacea	Immune & liver concerns	7 days
Green tea extract (capsules)	Bleeding risk at high doses	7 days

Medications to Keep Taking

Most chronic medications should be continued. Common ones to **keep taking** (with a small sip of water on surgery morning):

- **Blood pressure medications** — usually continue (except ACE inhibitors and ARBs — ask your surgeon)

- **Heart medications** (beta blockers, statins) — continue
- **Thyroid medication** — continue
- **Anti-seizure medications** — continue
- **Asthma inhalers** — continue (bring to hospital)
- **Antidepressants / Anti-anxiety** — usually continue (discuss MAOIs (a type of older antidepressant, such as phenelzine or tranylcypromine) specifically)

> **What About Metformin?**
>
> Metformin for diabetes is usually held the morning of surgery and for 48 hours after, especially if you're receiving IV contrast dye. Your surgeon will confirm.

COSMETIC SURGERY NOTE

Blood clot risk: Cosmetic procedures — especially abdominoplasty (tummy tuck), BBL, and combined procedures — carry an elevated risk for blood clots in the legs (DVT) or lungs (PE). Your surgeon may recommend:

- **Stop birth control pills or HRT** 4–6 weeks before surgery (estrogen increases clot risk)
- Blood clot prevention with compression stockings and/or low-dose blood thinners post-op
- Early and frequent walking after surgery

BARIATRIC SURGERY NOTE

Vitamin deficiency screening: Before bariatric surgery, your surgeon should order labs to check for existing deficiencies — because after surgery, absorption changes make these even harder to correct. Key labs include:

- Vitamin D (25-hydroxy)
- Vitamin B12

- Iron panel (ferritin, TIBC — total iron-binding capacity)
- Thiamin (vitamin B1)
- Folate
- Calcium
- Zinc

If any are low, your surgeon will correct them *before* surgery. This is especially important because post-op, your ability to absorb these nutrients will be permanently reduced.

GYNECOLOGIC SURGERY NOTE

Hormonal medications: Discuss all hormonal medications with your surgeon:

- **Birth control pills (combined oral contraceptives):** Slightly increase blood clot risk, but stopping before surgery is NOT routinely recommended. Blood clot prevention measures will be used.
- **Hormone replacement therapy (HRT):** Discuss with your surgeon — may or may not need to be paused depending on your risk factors
- **GnRH agonists (Lupron):** If you're on these for fibroids or endometriosis, your surgeon will guide timing

UTI screening: Urinary tract infections should be treated *before* surgery. If you have symptoms (burning, frequency, urgency), tell your surgical team right away.

C-SECTION NOTE

Prenatal vitamins: Continue your prenatal vitamins through delivery and postpartum, especially if breastfeeding. Additionally:

- Review your blood pressure medications with your OB team
- If on blood thinners (e.g., for blood clotting disorders), your OB will plan the timing carefully around delivery

- Anti-nausea medication can be given with your spinal/epidural to prevent nausea during surgery

CANCER SURGERY NOTE

Chemotherapy & medications: If you're receiving or have recently completed chemotherapy, your surgical and medical oncology teams will coordinate timing carefully:

- Some chemo drugs affect blood clotting, immune function, or wound healing
- Bring a complete list of all cancer medications, including targeted therapies and immunotherapies
- Blood thinners may need careful coordination around surgery
- Your blood counts will be checked before surgery to ensure safe levels

IF YOU HAVE DIABETES

Key medication guidance for diabetic patients:

- **Metformin:** Hold the day of surgery. Restart when eating normally.
- **SGLT2 inhibitors** (Jardiance, Farxiga, Invokana): **Stop 3–4 days before surgery** — risk of diabetic ketoacidosis under anesthesia.
- **Insulin:** Your doctor will adjust your dose — do NOT take your full morning dose while fasting. Typically take 50–80% of long-acting insulin the night before.
- **Sulfonylureas** (glipizide, glyburide): Hold the morning of surgery (risk of low blood sugar).
- **A1C goal:** Below 8% before elective surgery (ideally below 7%).
- **Recovery:** Check blood sugar every 4–6 hours. Target 140–180 mg/dL. Report readings above 250. Watch wounds closely — high blood sugar slows healing.

IF YOU HAVE HEART DISEASE

- **Beta-blockers** (metoprolol, carvedilol): **Continue** — do NOT stop suddenly. Take with a sip of water morning of surgery.
- **Statins** (atorvastatin, rosuvastatin): **Continue** through surgery.
- **ACE inhibitors** (-pril meds) and **ARBs** (-sartan meds): Often held the morning of surgery.
- **Diuretics:** Often held morning of surgery to prevent dehydration.
- **If you have a coronary stent:** Bare metal stent: wait 30+ days for elective surgery. Drug-eluting stent: wait 3–12 months. NEVER stop antiplatelet medications without your cardiologist's approval.
- Bring your pacemaker/defibrillator card if applicable. Bring recent cardiac test results (EKG, echo, stress test).

IF YOU TAKE BLOOD THINNERS

- **Warfarin:** Stop 5 days before. INR checked before surgery. May need bridging with injectable blood thinner.
- **Eliquis/Xarelto/Edoxaban:** Stop 2–3 days before (high bleeding risk) or 1–2 days (low risk).
- **Plavix:** Stop 5 days. **Brilinta:** Stop 5 days. **Effient:** Stop 7 days.
- **Aspirin:** Often continued — discuss with your surgeon.
- NEVER stop blood thinners without instructions from both your prescribing doctor AND your surgeon.

IF YOU HAVE LUNG DISEASE

- Practice incentive spirometer 10 times every hour while awake — start NOW, not after surgery.
- **Bring all inhalers** to the hospital. Use your rescue inhaler morning of surgery if instructed.
- If on inhaled steroids, continue through surgery.

- Walking and breathing exercises for 4+ weeks before surgery significantly reduces lung complications.
- After surgery: deep breathing every 1–2 hours, sit upright, walk early and often, cough and clear secretions.

IF YOU HAVE SLEEP APNEA

- **Bring your CPAP/BiPAP machine**, mask, headgear, and power cord to the hospital.
- Share your pressure settings and bring sleep study results if available.
- Continue using CPAP every night leading up to surgery.
- After surgery: you will use CPAP in recovery. Opioid doses will be carefully managed.
- Avoid sleeping flat on your back — keep head of bed elevated.

☑ Medication Review Checklist

- [ ] Made a list of ALL medications, supplements, vitamins, and herbals I take
- [ ] Confirmed blood thinner stop date with prescribing doctor
- [ ] Informed surgeon about GLP-1 medication (if applicable)
- [ ] Stopped all supplements and herbals 7–14 days before surgery
- [ ] Know which medications to take morning of surgery with a small sip of water
- [ ] Brought medication list to pre-op appointment

2

Nutrition & Fasting Guidelines

Fuel your body for healing — and the truth about fasting

What you eat in the weeks before surgery — and what you eat (or don't eat) the night before — directly affects your healing, your energy levels, and even your infection risk. Let's clear up one of the biggest myths in surgery prep first.

The Midnight Fasting Myth

You've probably heard: *"Nothing to eat or drink after midnight."* For decades, this was standard. But modern evidence tells a different story.

Current ASA 2023 fasting guidelines:

What	Minimum Fasting Time	Examples
Clear liquids	2 hours	Water, apple juice (no pulp), black coffee, tea, Gatorade, Pedialyte
Breast milk	4 hours	For infants
Light meal / non-human milk	6 hours	Toast, crackers, formula
Full meal (fatty/fried)	8+ hours	Steak, fried foods, heavy meals

Dr. Oluwole's Tip

Here's what many patients don't realize: being *dehydrated* before surgery is actually harmful. Drinking clear liquids up to 2 hours before arrival can improve your comfort, reduce nausea, stabilize your blood pressure, and make your IV easier to start. Don't suffer through unnecessary thirst. Follow your surgeon's specific instructions, but know the science supports hydration.

Carbohydrate Loading

ERAS (Enhanced Recovery After Surgery) protocols recommend drinking a clear carbohydrate drink 2–3 hours before surgery. This helps your body manage blood sugar more effectively, preserves muscle, and helps you feel better after surgery.

- **The night before:** 800mL of a clear carbohydrate drink (like ClearFast or Gatorade)
- **Morning of surgery:** 400mL, finished at least 2 hours before arrival
- *Note: Diabetic patients — discuss carb loading with your surgical team first*

Protein Targets for Healing

Your body needs protein to repair tissues, fight infection, and rebuild strength. In the 2–4 weeks before surgery, aim for:

- **1.2–1.5 grams of protein per kilogram of body weight per day**
- For a 150-lb (68 kg) person: **82–102 grams of protein daily**
- Spread intake across 4–5 meals/snacks

High-protein food sources:

Food	Protein (approx.)
Chicken breast (4 oz)	35g
Salmon (4 oz)	25g
Greek yogurt (1 cup)	17–20g
Eggs (2 large)	12g
Lentils (1 cup cooked)	18g
Cottage cheese (1 cup)	28g
Whey protein shake	20–30g
Tofu, firm (1/2 cup)	10g

Healing Nutrients

- **Vitamin C** (citrus, strawberries, bell peppers) — wound healing, collagen formation
- **Zinc** (meat, shellfish, pumpkin seeds) — immune function, tissue repair
- **Vitamin A** (sweet potatoes, carrots, leafy greens) — cell growth, immune support
- **Iron** (red meat, spinach, lentils) — oxygen transport, energy
- **Vitamin D** (sunlight, fortified foods, supplements) — bone healing, immune function

Foods to Avoid Before Surgery

- **Alcohol** — stop at least 48 hours before (ideally 1–2 weeks); affects bleeding, liver function, and anesthesia
- **Excessive sodium** — contributes to swelling and fluid retention
- **Processed sugar** — impairs immune function and wound healing
- **Fried and greasy foods** — harder to digest, increase nausea risk

BARIATRIC SURGERY NOTE

Pre-Op Diet: Most bariatric surgeons require a 2–4 week pre-op liver-shrinking diet. This is not optional — it reduces liver size so the surgeon can safely access your stomach.

- Typically high-protein, low-carb, low-fat
- Often includes protein shakes as meal replacements
- Your surgeon will provide specific instructions

Vitamin Corrections: If pre-op labs show deficiencies, begin correcting now:

- **Vitamin D:** 50,000 IU weekly if deficient
- **Iron:** ferrous sulfate 325mg with vitamin C for absorption
- **B12:** sublingual or injection if low

- **Thiamin (B1):** critical — deficiency can cause serious neurological problems post-bariatric surgery

GYNECOLOGIC SURGERY NOTE

Iron & anemia: Many gynecologic patients have low iron from heavy menstrual bleeding. If you're anemic:

- Your surgeon may prescribe iron supplements before surgery
- Severe anemia may need to be corrected before elective surgery can proceed
- Eat iron-rich foods: red meat, spinach, lentils, fortified cereals
- Take iron with vitamin C for better absorption; avoid taking with calcium or coffee

Bowel prep: For most gynecologic surgeries, routine bowel preparation is NOT needed. Your surgeon will tell you if it's required for your specific procedure.

C-SECTION NOTE

Eating before a scheduled C-section: Modern guidelines allow clear liquids up to 2 hours before your scheduled cesarean. Your OB team will give you specific fasting instructions.

Post-delivery nutrition: You can usually eat a regular diet within 2 hours of your C-section. Eating soon after surgery helps your bowels recover faster and supports breastfeeding.

CANCER SURGERY NUTRITION

Nutrition is critical for cancer patients: Malnutrition is common and significantly increases surgical complications.

- If you've lost weight unintentionally, tell your surgical team immediately
- You may be referred to a dietitian for a personalized plan

- **Immunonutrition** formulas (containing arginine and omega-3 fatty acids) may be recommended before major abdominal cancer surgery
- Oral nutritional supplements (Ensure, Boost) can help if appetite is poor
- Protein needs are even higher for cancer patients — aim for 1.5 g/kg daily

☑ Nutrition Checklist

- [ ] Increased protein intake to 1.2–1.5 g/kg daily
- [ ] Added healing nutrients (vitamin C, zinc, vitamin A) to my diet
- [ ] Stopped alcohol at least 48 hours before surgery
- [ ] Understand my fasting instructions (clear liquids 2 hrs, solids 6 hrs)
- [ ] Purchased carbohydrate loading drink (if recommended)

3

Prehabilitation

Train your body before surgery for faster recovery

Prehabilitation — "prehab" — is one of the most evidence-backed strategies to improve surgical outcomes. Patients who exercise before surgery recover faster, have fewer complications, shorter hospital stays, and less pain. Think of it like training for an athletic event — surgery is the event, and your body needs to be ready.

Dr. Oluwole's Tip

I tell my patients: surgery is a stress test for your body. The stronger you go into it, the stronger you come out. Even 2–3 weeks of consistent exercise makes a measurable difference.

Aerobic Exercise

Goal: 30 minutes, 3–5 times per week

Aerobic exercise improves your cardiovascular fitness, lung capacity, and circulation — all of which matter under anesthesia and during recovery.

- **Beginner:** Walking (brisk pace), stationary bike, water aerobics
- **Intermediate:** Jogging, cycling, swimming, elliptical
- **Advanced:** Running, HIIT intervals, rowing

Always start at your current fitness level and gradually increase. If you haven't exercised recently, walking is the single best exercise you can do.

Strength Training

Goal: 2–3 sessions per week

Muscle mass protects you during recovery. The more muscle you have going into surgery, the less strength you'll lose during your recovery period.

- **Upper body:** Wall push-ups, resistance band rows, overhead press (light weights)

- **Lower body:** Squats (bodyweight or light), lunges, step-ups, calf raises
- **Core:** Planks, bird-dogs, bridges — these support your spine and help with post-op mobility

Breathing Exercises & Incentive Spirometry

After surgery, your breathing will be shallower due to pain, sedation, and reduced mobility. Practicing deep breathing now trains your lungs and reduces your risk of pneumonia.

- **Incentive spirometer:** Use 10 times every hour while awake. Inhale slowly, hold 3–5 seconds, exhale. (Your surgeon may give you one at your pre-op visit; they're also ~$10 at any pharmacy.)
- **Diaphragmatic breathing:** Place hand on belly. Inhale through nose for 4 counts (belly rises), hold 4 counts, exhale through pursed lips for 6 counts. Repeat 10 times, 3x daily.
- **Coughing exercise:** Practice splinted coughing (hold a pillow against your incision area). This will be important post-op.

ORTHOPEDIC SURGERY FOCUS

Pre-op exercises for joint surgery:

- **Sit-to-stand practice:** From a chair, stand without using hands. 10 reps, 3x daily — this mimics what you'll need to do immediately after knee or hip surgery
- **Quad sets:** Sitting with legs straight, tighten thigh muscles and hold 5 seconds. 20 reps, 3x daily
- **Straight leg raises:** Lying down, lift leg 12 inches, hold 5 seconds. 10 reps each leg
- **Balance training:** Stand on one leg (near a counter for safety) for 30 seconds. Switch sides. Repeat 5x
- **Ankle pumps:** Flex and point your feet 20 times every hour — start practicing now and continue post-op for blood clot prevention

COSMETIC / ABDOMINAL SURGERY NOTE

Core preparation: Strengthening your core before abdominoplasty, liposuction, or any abdominal procedure helps post-op mobility. However:

- Avoid heavy sit-ups/crunches in the final 2 weeks before surgery
- Focus on gentle stabilization: planks, pelvic tilts, bird-dogs
- Walking and cardio are your best friends for overall readiness

BARIATRIC SURGERY NOTE

If you're starting from a sedentary baseline, that's completely okay. Your prehab program may look different — and that's normal. Start with:

- Walking 10–15 minutes daily, gradually increasing
- Chair exercises if needed
- Pool walking or water aerobics (excellent for joint-friendly cardio)
- Focus on building the habit now — exercise will be a lifelong part of your bariatric success

GYNECOLOGIC SURGERY FOCUS

Pelvic floor preparation: Strengthening your pelvic floor before gynecologic surgery can improve post-op outcomes:

- **Kegel exercises:** Contract pelvic floor muscles (as if stopping urine flow), hold 5–10 seconds, relax. 10 reps, 3x daily
- **Walking:** 20–30 minutes daily improves circulation and overall conditioning
- **Diaphragmatic breathing:** Coordinating breath with pelvic floor relaxation helps with post-op recovery

- **Core stabilization:** Gentle planks and pelvic tilts (avoid heavy core work if you have large fibroids or prolapse)

C-SECTION PREPARATION

Staying active during pregnancy: If your OB has cleared you for exercise, maintaining activity before a planned cesarean can improve recovery:

- Walking is the safest and most effective exercise in late pregnancy
- Prenatal yoga and stretching maintain flexibility
- Pelvic floor exercises (Kegels) help even with cesarean delivery
- Practice the breathing exercises described above — they'll help post-op

CANCER SURGERY PREHABILITATION

Prehabilitation can reduce cancer surgery complications by up to 40%. Start as soon as surgery is planned:

- Aim for 30 minutes of moderate activity most days — even small amounts help
- If you're receiving neoadjuvant chemo/radiation, prehab during this time is especially valuable
- **If you smoke:** Quitting before surgery is one of the most impactful things you can do. Four weeks of abstinence significantly reduces complications. Nicotine replacement is safe and is NOT a reason to delay surgery.
- Breathing exercises are especially important before lung or upper abdominal cancer surgery
- Mental health support is critical — distress is highest around diagnosis and surgery

Sample Weekly Prehab Schedule

Day	Beginner	Intermediate	Advanced
Mon	Walk 20 min	Walk/jog 30 min	Run 30 min + weights
Tue	Chair exercises 15 min	Light weights 20 min	Strength training 30 min
Wed	Walk 20 min	Swim/cycle 30 min	HIIT 25 min
Thu	Rest + breathing exercises	Rest + breathing exercises	Active recovery + yoga
Fri	Walk 25 min	Walk/jog 30 min	Run 30 min + weights
Sat	Chair exercises 15 min	Light weights 20 min	Strength training 30 min
Sun	Gentle walk + stretching	Gentle walk + stretching	Rest + stretching

Every level: practice breathing exercises (incentive spirometry or diaphragmatic breathing) 3 times daily.

☑ Prehab Checklist

- [ ] Started aerobic exercise routine (walking counts!)
- [ ] Incorporated strength training 2–3x per week
- [ ] Practicing breathing exercises daily
- [ ] Purchased or received an incentive spirometer

4

Mental Preparation

Managing anxiety and setting expectations

Feeling nervous before surgery is completely normal. In fact, research shows that up to 80% of patients experience significant anxiety before their procedure. The good news? Mental preparation is a skill you can practice — and it genuinely improves outcomes. Patients who feel mentally prepared report less pain, need less medication, and recover faster.

Dr. Oluwole's Tip

I have been on both sides — as the anesthesiologist putting patients to sleep and as someone who understands what it feels like to trust your life to a team of people you may have just met. Your feelings are valid. What I can tell you is this: we have trained for years for exactly this moment. Our entire job is keeping you safe.

Managing Surgical Anxiety

Evidence-based strategies that work:

- **Education reduces fear.** The fact that you're reading this masterclass means you're already doing something powerful. Fear of the unknown is the biggest driver of surgical anxiety.
- **Name your specific fears.** "I'm scared of surgery" is harder to manage than "I'm worried about waking up during surgery" (which we can address directly — awareness under anesthesia is extremely rare, approximately 1 in 19,000 cases).
- **Limit Dr. Google.** After a certain point, online research increases anxiety rather than reducing it. You have this guide. Trust your surgical team.
- **Talk to someone.** A therapist, a friend, a family member, or our mental wellness counselor Stacey J. Floyd, MA, LPC.
- **Physical activity.** Exercise is one of the most effective anti-anxiety interventions. (See Module 3.)
- **Sleep hygiene.** Anxiety worsens with poor sleep. Maintain a consistent schedule, limit screens before bed, and avoid caffeine after noon.

Guided Visualization Exercise

This 7-step visualization can be done daily in the week before surgery. Find a quiet place, close your eyes, and walk through each step for 1–2 minutes:

1. **Arriving at the hospital:** Picture yourself walking in calmly, checking in, feeling prepared.
2. **Pre-op area:** You're in a warm gown, your IV is placed smoothly, your nurse is kind.
3. **Meeting your anesthesiologist:** They explain the plan. You feel heard and safe.
4. **Going to sleep:** You take a deep breath, the medication feels warm, and you drift peacefully.
5. **Waking up:** You open your eyes. It's over. You feel relieved. The team is there.
6. **Recovery room:** You're resting comfortably. Pain is managed. Your loved one is nearby.
7. **Going home:** You're in your own bed, healing, getting stronger each day.

> **Dr. Oluwole's Tip**
>
> Visualization isn't "woo-woo" — it's a tool used by elite athletes, military personnel, and surgeons. When you mentally rehearse a positive outcome, your nervous system responds as if it's already happening. This reduces cortisol (stress hormone) and primes your body for recovery.

10 Questions to Ask Your Surgeon

1. What are the specific risks of my procedure?
2. How many times have you performed this surgery?
3. What is the expected recovery timeline?
4. What are the signs of complications I should watch for?
5. How will my pain be managed after surgery?
6. When can I return to work / driving / exercise?

7. Are there any alternatives to surgery I should consider?
8. What should I do to prepare in the weeks before?
9. What medications should I stop or continue?
10. Who do I call if I have a problem after discharge?

8 Questions to Ask Your Anesthesiologist

1. What type of anesthesia will I have?
2. How will my nausea be prevented? (Especially if you have a history of it.)
3. I take [medication/supplement] — does that affect my anesthesia?
4. Will I have a breathing tube?
5. How will my pain be controlled during and after surgery?
6. What are the risks of anesthesia for me specifically, given my health history?
7. How long will it take to fully wake up?
8. Is there anything I can do to reduce my anesthesia risks?

Setting Realistic Recovery Expectations

One of the biggest sources of post-op frustration is mismatched expectations. Here's what to realistically expect:

- **You will not feel like yourself immediately.** Fatigue, brain fog, and emotional swings are normal for 1–4 weeks.
- **Recovery is not linear.** You'll have good days and setbacks. Both are normal.
- **Pain does not mean something is wrong.** Surgical pain is expected. Uncontrolled, worsening pain IS a concern.
- **Asking for help is strength, not weakness.** You will need a caregiver. Plan for it now.
- **Social media recovery timelines are misleading.** No one posts their worst days. Healing takes time.

☑ Mental Prep Checklist

- [ ] Identified and named my specific fears about surgery
- [ ] Practiced the guided visualization exercise at least once
- [ ] Prepared my list of questions for the surgeon
- [ ] Prepared my list of questions for the anesthesiologist
- [ ] Talked to someone about how I'm feeling
- [ ] Set realistic expectations for my recovery timeline

Dr. Oluwole's Tip

Want personalized, one-on-one guidance? Dr. Oluwole offers virtual Pre-Surgical Consultations where she reviews your complete health picture and builds a preparation plan specifically for your surgery.

Book your consultation → (https://www.opwellconcierge.com/schedule)

5

Day-of-Surgery Checklist

Everything you need the night before and morning of

The Night Before

- Shower with antibacterial soap (Hibiclens/chlorhexidine if instructed). Wash from neck to toes. Do not apply lotion, deodorant, or perfume after.
- Eat a light, easy-to-digest dinner (no heavy, fried, or fatty foods)
- Lay out comfortable, loose-fitting clothes for tomorrow (button/zip-front top is ideal)
- Pack your hospital bag (see packing list below)
- Set two alarms — you don't want to be rushed
- Charge your phone
- Remove nail polish (the anesthesia team monitors your nail beds for oxygen levels)
- Practice your breathing exercises one final time
- Do your visualization exercise (Module 4)

What to Pack

💼 Hospital Bag Checklist

- [ ] Photo ID and insurance cards
- [ ] List of all medications with dosages
- [ ] Advance directive / living will (if applicable)
- [ ] Comfortable, loose-fitting going-home outfit
- [ ] Slip-on shoes (no laces — you won't want to bend over)
- [ ] Phone charger
- [ ] CPAP machine (if you use one for sleep apnea)
- [ ] Glasses / contacts case (remove contacts before surgery)
- [ ] Comfort item (small pillow for seatbelt on ride home)
- [ ] This masterclass (printed or on phone) for reference

Surgery Morning

☑ Morning-of Checklist

- [ ] Shower with antibacterial soap again (if instructed for 2nd wash)
- [ ] No makeup, lotion, deodorant, perfume, or hair products
- [ ] No jewelry, piercings, or hair accessories (leave all at home)
- [ ] Took approved morning medications with a small sip of water
- [ ] Finished carbohydrate drink 2 hours before arrival (if instructed)
- [ ] Confirmed ride home with a responsible adult
- [ ] Arrived on time (usually 1.5–2 hours before scheduled surgery)

What to Tell the Anesthesia Team at Check-In

When you meet your anesthesiologist (usually 30–60 minutes before surgery), tell them about:

- Any allergies (medications, latex, food — especially eggs and soy)
- Previous anesthesia problems (nausea, difficult airway, family history of malignant hyperthermia)
- When you last ate or drank (be honest — this is a safety issue)
- All medications taken today
- Loose or capped teeth, dental work
- Sleep apnea or CPAP use
- Any cold, cough, or illness in the past 2 weeks
- Pregnancy possibility

Dr. Oluwole's Tip

Please be completely honest with your anesthesia team. We are not there to judge you — we are there to keep you safe. If you ate something you weren't supposed to, tell us. If you took a supplement you forgot to stop, tell us. We can adjust our plan, but only if we know the truth.

COSMETIC SURGERY REMINDERS

- **No makeup, no nail polish** (acrylic or gel nails must be removed from at least 2 fingers)
- **No lotions, creams, or self-tanner** (interferes with surgical markers and monitoring)
- **Compression garments ready** at home for when you return (your surgeon will tell you which kind)
- Wear a zip-front or button-front top — you will not be able to raise your arms overhead after many cosmetic procedures

GENERAL SURGERY NOTE

Bowel preparation: For some abdominal surgeries, your surgeon may prescribe a bowel preparation. Follow the instructions exactly — the timing matters. If no bowel prep was prescribed, you do not need one.

BARIATRIC SURGERY REMINDERS

- Bring your **vitamin and supplement schedule** to review with your team
- Have your **post-op diet plan** printed or saved on your phone (clear liquids ready at home)

- Pack a small pillow for the car ride — the seatbelt will be uncomfortable against your abdomen

GYNECOLOGIC SURGERY REMINDERS

- **Do NOT shave** the surgical area yourself — this increases infection risk. The surgical team will handle hair removal with clippers if needed.
- **Treat infections first:** If you have a UTI or vaginal infection, tell your team — elective surgery should wait until it's treated
- Bring **loose, comfortable clothing** and underwear for going home — avoid tight waistbands
- Bring **maxi pads** (not tampons) — you may have vaginal bleeding after surgery

C-SECTION DAY REMINDERS

- **Car seat:** Make sure the infant car seat is installed before surgery day
- **Hospital bag:** Pack for yourself and baby (going-home outfits, nursing bra, high-waisted underwear that won't rub the incision)
- You will likely receive **spinal or epidural anesthesia** — you'll be awake during delivery
- Your support person can usually be present in the operating room
- **Skin-to-skin contact** can often happen in the OR shortly after delivery

CANCER SURGERY DAY REMINDERS

- Bring your **complete medication list** including all cancer treatments
- Bring your **advance directive** if you have one

- Know that the surgery may take longer than expected if the surgeon finds more disease than anticipated
- Pathology results will take several days to a week+ — be prepared for this waiting period
- You may wake up with **drains, tubes, or other devices** — your team will explain each one

6

Recovery Roadmap

Week-by-week expectations for your healing journey

Recovery is a journey, not a single moment. Understanding what to expect each week helps you recognize normal healing and catch anything that needs attention. Here is your general roadmap — your surgeon will provide specifics for your procedure.

Week 1: The Hardest Part

This is the most challenging week. Your body is in acute healing mode.

- **Pain:** Peaks at days 2–3, then gradually improves. Pain at the incision site is expected.
- **Fatigue:** Profound tiredness. Anesthesia can take 24–72 hours to fully clear. You may sleep 12–16 hours a day.
- **Swelling & bruising:** Usually worst at days 3–5 before improving
- **Appetite:** May be poor. Focus on fluids and small, frequent meals.
- **Mood:** Feeling emotional, weepy, or irritable is very common. Anesthesia, pain medications, and the stress of surgery all affect your mood.
- **Activity:** Short walks (even just to the bathroom and back) starting day 1. This is the single most important thing you can do to prevent blood clots and pneumonia.

> **Dr. Oluwole's Tip**
>
> Week 1 is not the time to be a hero. Rest, accept help, take your medications on schedule, and walk a little bit every day. That's it. Everything else can wait.

Week 2: Turning the Corner

- **Pain:** Significantly better. Many patients transition off prescription pain medication.
- **Energy:** Starting to return, but you'll still tire easily. Don't push it.

- **Incision:** Should be healing with no active drainage or spreading redness
- **Activity:** Longer walks. Light household tasks. No lifting over 10 lbs (typically).
- **Follow-up:** Most surgeons schedule a 1–2 week post-op visit

Weeks 3–4: Building Back

- **Pain:** Mostly managed with over-the-counter medication (acetaminophen, ibuprofen if cleared)
- **Energy:** Noticeably improving. Still need more rest than normal.
- **Activity:** Gradually increasing. Walking 20–30 minutes. Some patients return to desk work.
- **Mood:** Stabilizing, though some patients experience a "post-op dip" around week 3–4 when initial relief wears off and frustration with limitations sets in

Normal vs. Concerning Symptoms

Normal	Concerning — Call Your Surgeon
Mild to moderate pain (improving daily)	Pain getting worse after day 3–4
Bruising and mild swelling	Increasing redness, warmth, or streaking from incision
Small amount of clear/pinkish drainage	Thick, cloudy, or foul-smelling drainage
Low-grade temperature (up to 100.3°F)	Fever above 100.4°F (38°C)
Fatigue and sleepiness	Confusion, difficulty staying awake
Mild constipation	No bowel movement for 4+ days despite treatment
Emotional ups and downs	Persistent depression, hopelessness, or thoughts of self-harm

Pain Management: The Multimodal Approach

Modern pain management uses multiple tools working together rather than relying solely on opioids:

1. **Acetaminophen (Tylenol):** Foundation of post-op pain control. 1000mg every 6–8 hours (max 3000mg/day) (your provider may approve up to 4000mg/day if you have no liver conditions and do not drink alcohol regularly). Take on a schedule, not "as needed."
2. **NSAIDs (ibuprofen/naproxen):** If cleared by your surgeon (not after certain surgeries). Alternate with acetaminophen.
3. **Ice therapy:** 20 minutes on, 20 minutes off. Reduces swelling and numbs pain.
4. **Nerve blocks / Regional anesthesia:** If used during surgery, may provide 12–72 hours of pain relief.
5. **Opioids (as rescue only):** Oxycodone, hydrocodone — use the lowest dose for the shortest time. (See Module 9 for details.)

BARIATRIC RECOVERY: DIET STAGES

After bariatric surgery, your diet progresses in carefully timed stages. Do not skip stages.

Stage	Timeline	Examples
Clear liquids	Days 1–2	Water, broth, sugar-free Jello, diluted juice
Full liquids	Days 3–14	Protein shakes, cream soups (strained), yogurt (smooth)
Pureed foods	Weeks 3–4	Scrambled eggs, hummus, mashed potatoes, pureed chicken
Soft foods	Weeks 5–6	Baked fish, soft fruit, steamed vegetables, ground meat
Regular diet	Week 7+	Normal foods, small portions, chew thoroughly

Lifetime vitamins: After bariatric surgery, you will need daily supplements *for life*:

- Bariatric multivitamin (2x daily or as directed)
- Calcium citrate with vitamin D (1200–1500mg/day in divided doses)
- Vitamin B12 (sublingual or injection)
- Iron (menstruating women especially)

COSMETIC SURGERY RECOVERY

Compression garments: Wear your compression garment 24/7 for the first 4–6 weeks (remove only for showering when cleared). Proper compression reduces swelling, supports your results, and improves comfort.

Swelling timeline:

- Weeks 1–2: Significant swelling (you will NOT see your final result)
- Weeks 3–6: Swelling gradually decreasing, shape emerging
- Months 3–6: Major improvements, most swelling resolved
- Month 12: Final result. Yes, it takes a full year for some procedures (especially BBL and tummy tuck)

Scar care: Begin silicone scar sheets or gel after incisions are fully closed (usually 2–3 weeks). Sun protection (SPF 30+) on scars for 1 year.

ORTHOPEDIC SURGERY RECOVERY

Physical therapy / Rehab: This is non-negotiable for joint and orthopedic surgery. Your PT exercises are just as important as the surgery itself.

- Begin PT exercises as directed (often day 1 after surgery)
- Outpatient PT typically starts 1–2 weeks post-op, 2–3 sessions per week

- Consistency matters more than intensity — do your home exercises daily

Blood clot prevention:

- Ankle pumps and calf squeezes every hour while awake
- Blood thinner medication as prescribed (usually 2–6 weeks)
- Compression stockings (TED hose) as instructed
- Walk as soon and as often as cleared

Weight bearing timeline (varies by procedure — follow YOUR surgeon's instructions):

- Total knee replacement: usually weight-bearing as tolerated immediately
- Total hip replacement: usually weight-bearing as tolerated with walker/cane
- ACL reconstruction: limited weight bearing for 2–6 weeks
- Fracture repair: varies widely — strictly follow surgeon's instructions

GENERAL SURGERY RECOVERY NOTE

Shoulder/back pain after laparoscopic surgery: After laparoscopic surgery, you may experience shoulder or upper back pain from the CO2 gas used to inflate your abdomen during the procedure. This is common, not dangerous, and usually resolves in 1–3 days. Walking and warm compresses on your shoulder can help.

GYNECOLOGIC SURGERY RECOVERY

Vaginal bleeding and discharge: Some vaginal bleeding or spotting is normal after hysterectomy, myomectomy, and other gynecologic procedures. This may last several weeks.

- Use maxi pads only — **no tampons** until your surgeon clears you (usually 6 weeks)

- **No douching**
- **Sexual activity:** Wait 6 weeks after hysterectomy; ask your surgeon for other procedures
- **Shoulder pain** is common after laparoscopic gynecologic surgery (from CO2 gas) — it resolves in 1–3 days

Emotional changes: Feelings of grief, relief, sadness, or mood swings are all normal after hysterectomy or oophorectomy. If ovaries were removed, you may experience sudden menopause symptoms (hot flashes, mood changes). Talk to your doctor about hormone replacement if needed.

C-SECTION RECOVERY TIMELINE

Your recovery is different because you're healing from major surgery while caring for a newborn:

- **Week 1:** Pain worst in first 2–3 days. Focus on rest, walking short distances, breastfeeding, and pain medication on schedule. Don't lift anything heavier than your baby.
- **Week 2:** Pain improving. Longer walks. Continue stool softeners. "Baby blues" (mood swings, tearfulness) are common.
- **Weeks 3–4:** Steadily improving. Most women off prescription pain medication. Incision should be healing well.
- **Weeks 4–6:** Postpartum checkup. Most women cleared for driving, exercise, and sexual activity.

Vaginal bleeding (lochia): You will have vaginal bleeding for 4–6 weeks after cesarean delivery — this is your uterus healing, not from the incision. Use pads only. Heavy bleeding (soaking more than 1 pad/hour) or foul-smelling discharge needs medical attention.

CANCER SURGERY RECOVERY

Returning to Intended Oncologic Therapy (RIOT): One of the goals of good recovery is being well enough to start additional cancer treatment (chemo/radiation) on time.

- Prehabilitation and ERAS protocols help you recover faster for next-phase treatment
- Tell your team if you're struggling with recovery so they can help
- **Drains:** You may go home with surgical drains. Learn to empty them, measure output, and recognize problems.
- **Pathology results:** Will take days to a week+. These determine your cancer type, grade, margins, and next steps.
- **Emotional recovery:** Relief, anxiety, sadness, fear of recurrence are all normal. Seek professional help if distress persists.

7

When to Call Your Doctor vs. Go to the ER

Know the difference — it could save your life

After surgery, it can be hard to know what's normal and what's not. This guide helps you make the right call. When in doubt, **always call your surgeon's office first** — they have an after-hours line for a reason.

📞 Call Your Surgeon

- Fever above 100.4°F (38°C)
- Increasing pain not relieved by medication
- Wound changes: increased redness, warmth, swelling, or drainage
- Foul-smelling wound drainage
- Can't keep fluids or medications down for 12+ hours
- No bowel movement for 4+ days
- Persistent nausea or vomiting beyond 48 hours
- New numbness or tingling near the surgical site
- Medication concerns or side effects

🚨 Go to the ER / Call 911

- Chest pain or pressure
- Severe difficulty breathing or sudden shortness of breath
- Heavy bleeding that won't stop with pressure
- Signs of blood clot in leg (calf pain, swelling, warmth, redness in one leg)
- Sudden severe headache or vision changes
- Confusion or difficulty staying conscious
- Allergic reaction (hives, swelling, difficulty breathing)
- Fever above 103°F (39.4°C)
- Fainting or passing out

DVT vs. PE: Know the Difference

Blood clots are one of the most serious post-surgical complications. They can form in your leg veins (DVT) and travel to your lungs (PE). Knowing the signs can save your life.

DVT (Deep Vein Thrombosis)	PE (Pulmonary Embolism)

Pain or cramping in one calf or thigh	Sudden shortness of breath
Swelling in one leg (not both)	Sharp chest pain (worse with deep breath)
Warmth in the affected area	Rapid heart rate
Redness or discoloration	Coughing (may cough up blood)
Leg feels heavy or tender	Feeling lightheaded or faint

CRITICAL

A PE is a medical emergency. If you experience sudden shortness of breath or chest pain after surgery, call 911 immediately. Do not drive yourself.

Reduce your blood clot risk:

- Walk as soon and as often as your surgeon allows
- Do ankle pumps every hour while sitting or lying down
- Wear compression stockings as instructed
- Take blood thinners if prescribed (do not skip doses)
- Stay hydrated
- Avoid sitting or lying in one position for more than 2 hours

MEDICAL TOURISM WARNING

If you traveled for surgery (especially internationally), blood clot risk is significantly elevated due to the combination of surgery + prolonged sitting during travel.

- **Do not fly for at least 7–14 days** after surgery (discuss with your surgeon)
- If you must travel, wear compression stockings, stay hydrated, and walk every 1–2 hours during the flight
- Have a plan for medical care at your destination AND at home
- Carry your surgical records and surgeon's contact information

- Know the nearest ER to your recovery location

GYNECOLOGIC SURGERY WARNING SIGNS

In addition to the general warning signs above, call your surgeon if you experience:

- **Heavy vaginal bleeding** (soaking more than one pad per hour)
- **Foul-smelling vaginal discharge**
- **Difficulty urinating** or inability to urinate
- **Increasing abdominal bloating or distension**
- **Persistent nausea or vomiting** that prevents you from keeping fluids down

C-SECTION WARNING SIGNS

In addition to the general warning signs above, seek medical attention for:

- **Heavy vaginal bleeding** (soaking more than one pad per hour) or large blood clots
- **Foul-smelling vaginal discharge**
- **Incision opening up** or signs of infection (redness, warmth, cloudy drainage)
- **Severe headache** that doesn't improve with medication (especially if you had spinal/epidural)
- **Signs of postpartum depression:** persistent sadness, inability to care for yourself or your baby, thoughts of harming yourself or your baby — these are medical emergencies
- **Breast redness, pain, or fever** (possible mastitis if breastfeeding)

CANCER SURGERY WARNING SIGNS

In addition to the general warning signs above, contact your team for:

- **Changes in drain output:** Sudden increase, change in color, or foul smell
- **Signs of low blood counts:** Easy bruising, prolonged bleeding, unusual fatigue (especially if recent chemotherapy)
- **Fever:** Especially important if you've had recent chemotherapy, as your immune system may be compromised
- **New lumps or swelling** near the surgical site
- **Persistent or worsening emotional distress:** Anxiety, depression, or difficulty coping

8

Understanding Anesthesia

Bonus Module — Types, the team, and what to expect

Anesthesia is one of the most common sources of fear before surgery — often because people don't fully understand what it involves. Let's demystify it.

Types of Anesthesia

Type	What It Does	Common Uses
Local	Numbs a small area. You're fully awake.	Skin biopsies, dental work, minor repairs
Regional — Nerve Block	Numbs a larger area (arm, leg, abdomen). You may be awake or lightly sedated.	Hand/arm surgery, knee arthroscopy
Spinal	Single injection in lower back. Numbs from waist down for 1–3 hours.	C-section, hip/knee replacement, hernia
Epidural	Catheter in lower back for continuous medication. Can be adjusted.	Labor & delivery, major abdominal surgery
Sedation (MAC)	"Twilight" — you're relaxed and drowsy but breathe on your own.	Colonoscopy, minor cosmetic procedures
General	Completely asleep. Breathing tube or mask. No awareness.	Major surgery, any procedure requiring complete stillness

The Anesthesia Team

- **Anesthesiologist (MD/DO):** Physician who plans your anesthetic, manages your vital signs, and handles emergencies. Board-certified after 4 years of specialized training after medical school.
- **CRNA (Certified Registered Nurse Anesthetist):** Advanced practice nurse with specialized anesthesia training. May work independently or with an anesthesiologist depending on the facility.

- **Anesthesiologist Assistant (AA):** Works under the direct supervision of an anesthesiologist.

> **Dr. Oluwole's Tip**
>
> You have the right to ask who will be providing your anesthesia and what their qualifications are. A good anesthesia team will welcome your questions — it shows you're engaged in your care.

What Happens When You "Go Under"

1. You'll breathe oxygen through a mask for a few minutes ("pre-oxygenation")
2. IV medication is given — you'll feel warm and relaxed, then sleepy
3. You'll fall asleep in about 15–30 seconds
4. A breathing tube or airway device is placed (you won't feel this)
5. Throughout surgery, your anesthesiologist monitors: heart rate, blood pressure, oxygen, breathing, temperature, and depth of anesthesia

Waking Up

- As surgery ends, anesthesia is gradually reduced
- The breathing tube is removed (you may have a sore throat for 1–2 days)
- You'll wake up in the recovery room (PACU) — it may feel like only seconds have passed
- Feeling groggy, confused, cold, or emotional is completely normal
- Nausea can occur — tell your nurse immediately; they have medications ready

Common Anesthesia Side Effects

- **Sore throat** (from breathing tube) — 1–3 days, mild
- **Nausea/vomiting** — common, usually treatable (see Module 12)
- **Shivering** — common, resolves quickly with warming blankets

- **Grogginess/confusion** — 24–72 hours to fully clear
- **Muscle aches** — from medications used during anesthesia, resolve in 1–2 days
- **Hoarse voice** — usually resolves within a week

9

Pain Management Deep Dive

Bonus Module — The modern approach to surgical pain

Effective pain management is one of the most important factors in your recovery. The goal is not zero pain — some discomfort after surgery is inevitable and actually signals healing. The goal is *manageable* pain that allows you to move, breathe deeply, eat, and sleep.

The Multimodal Approach

Modern pain management layers multiple strategies together. Each one attacks pain through a different mechanism, so you get better relief with fewer side effects than relying on any single medication.

Step 1: Non-Opioid Foundation

- **Acetaminophen (Tylenol):** 1000mg every 6–8 hours. Take on a SCHEDULE, not "as needed." This is the single most important pain medication after surgery.
- **Ibuprofen (Advil/Motrin):** 400–600mg every 6–8 hours WITH FOOD. *Only if cleared by your surgeon.* Alternating with acetaminophen provides excellent coverage.
- **Naproxen (Aleve):** 220–440mg every 12 hours. Alternative to ibuprofen.

> **Alternating Schedule Example**
>
> 8 AM: Tylenol 1000mg → 11 AM: Ibuprofen 400mg → 2 PM: Tylenol 1000mg → 5 PM: Ibuprofen 400mg → 8 PM: Tylenol 1000mg → 11 PM: Ibuprofen 400mg. This provides near-continuous pain relief.

Step 2: Adjunct Therapies

- **Ice therapy:** 20 minutes on, 20 off. Wrap ice pack in a thin towel — never directly on skin or incision.
- **Elevation:** Keep surgical area elevated above heart level when possible
- **Nerve blocks:** If placed during surgery, may provide 12–72 hours of excellent pain relief. When the block wears off, start your oral medications BEFORE the pain arrives.

- **Gabapentin/pregabalin:** Sometimes prescribed to reduce nerve pain and opioid needs

Step 3: Opioids (Rescue Only)

Opioids (oxycodone, hydrocodone, tramadol) are reserved for breakthrough pain that non-opioid medications can't control.

- Use the **lowest effective dose**
- Most patients need opioids for **3–5 days or less**
- Take with a stool softener (constipation is almost guaranteed — see Module 13)
- Do NOT drive, operate machinery, or make important decisions while taking opioids
- Do NOT mix with alcohol, benzodiazepines (Xanax, Valium), or sleep aids

Opioid Weaning Schedule

Don't stop opioids abruptly if you've been taking them for more than 3–5 days. A gradual taper prevents withdrawal:

- Days 1–3: Take as prescribed for breakthrough pain
- Days 4–5: Reduce to half dose, use only at night or for activity
- Days 5–7: Switch to acetaminophen + ibuprofen only
- If you're still needing opioids after 7–10 days, call your surgeon

Safe Storage & Disposal of Opioids

- **Store** in a locked location, away from children, teens, and visitors
- **Count** your pills — know how many you have
- **Never share** your prescription with anyone
- **Dispose** of unused pills at a pharmacy take-back program or mix with coffee grounds/cat litter in a sealed bag and throw away
- The DEA holds National Prescription Take-Back events twice a year

OPIOID SAFETY

Signs of opioid overdose: extreme drowsiness, slow/shallow breathing, blue lips or fingertips, unresponsiveness. If you see these signs in anyone, call 911 immediately and administer naloxone (Narcan) if available.

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Preventing Infection

Bonus Module — Evidence-based strategies to protect your incision

Surgical site infections (SSIs) occur in about 2–5% of surgeries. The good news: many are preventable with simple, evidence-based steps that start before you even get to the hospital.

Pre-Op Showering Protocol

- **Night before AND morning of surgery:** Shower with chlorhexidine (Hibiclens) soap if instructed. Wash from the neck down. Let the soap sit on your skin for 2 minutes before rinsing.
- Do NOT shave the surgical area. If hair removal is needed, the surgical team will use electric clippers (not razors — razors cause micro-cuts that harbor bacteria).
- Do NOT apply lotion, deodorant, perfume, or powder after your pre-op shower
- Use clean towels and put on clean clothes

Preventing Staph Bacteria Before Surgery

Some surgeons prescribe a preventive nose ointment protocol (prescription antibiotic mupirocin ointment inside both nostrils, 2x daily for 5 days before surgery) to eliminate Staph bacteria. This is especially common before:

- Joint replacement surgery
- Cardiac surgery
- Spine surgery with hardware
- Any implant-based procedure

Blood Sugar Control

Elevated blood sugar significantly increases infection risk — even in patients without diabetes. If you have diabetes:

- Optimize your blood sugar control in the weeks before surgery

- Target A1C below 8% (ideally below 7%) before elective surgery
- Your surgical team will monitor blood sugar closely during and after surgery

Post-Op Wound Care

- **Keep the dressing clean and dry** for the first 24–48 hours (or as instructed)
- **Wash hands** thoroughly before and after touching near your incision
- **Do not submerge** your incision in water (no baths, pools, hot tubs) until fully healed (usually 2–4 weeks)
- **Gentle shower** is usually okay after 48 hours — let water run over the incision, pat dry. Do not scrub.
- **Do not pick at** scabs, Steri-Strips, or surgical glue — let them fall off naturally

Warning Signs of Infection

- Increasing redness spreading from the incision
- Warmth and swelling that worsens after day 3–5
- Pus or thick, cloudy, or foul-smelling drainage
- Fever above 100.4°F
- Red streaks extending from the wound
- The wound opening up (dehiscence)

Dr. Oluwole's Tip

The single best thing you can do to prevent infection? Wash your hands. Every time. Before and after touching your incision area. It sounds simple because it is — and it works.

GENERAL SURGERY NOTE

Surgical drain care: If you go home with a surgical drain, your care team will show you how to empty and measure the output. Keep the drain site clean, secure the drain so it doesn't pull, and record the amount each time you empty it.

11

The Recovery Room (PACU)

Bonus Module — What to expect when you wake up

The PACU (Post-Anesthesia Care Unit) is where you'll wake up after surgery. Knowing what to expect removes one more source of anxiety.

What PACU Looks Like

The recovery room is usually a large, open area with curtained bays. It's brightly lit, and you'll hear monitors beeping. There will be nurses nearby at all times. It can feel disorienting at first, and that's completely normal.

What's Connected to You

- **IV line:** In your hand or arm for fluids and medications
- **Pulse oximeter:** Small clip on your finger measuring oxygen levels
- **Blood pressure cuff:** Will inflate automatically every few minutes
- **Heart monitor:** Small stickers on your chest with wires (no pain)
- **Oxygen:** Nasal cannula (small tubes in your nose) or face mask
- **Urinary catheter:** For longer surgeries (you may or may not have one)
- **Surgical drain:** Small tube near your incision (depends on surgery type)

The Waking-Up Process

You may feel:

- **Confused or disoriented** — "Where am I? Is it over?" This is extremely common.
- **Cold** — Operating rooms are kept cool. Warm blankets will be provided.
- **Emotional** — Crying, laughing, or feeling anxious upon waking is normal. Anesthesia affects the brain's emotional centers.
- **Thirsty** — Your nurse will start with ice chips, then small sips of water.
- **Nauseated** — Tell your nurse immediately. Anti-nausea medication works fast.

- **Pain** — Your nurse will ask your pain level (0–10). Tell them honestly so they can treat it.

How Long You'll Stay

- **Outpatient surgery:** 1–3 hours in PACU before discharge home
- **Inpatient surgery:** 1–2 hours in PACU, then transferred to your hospital room

Discharge Criteria

Before you can go home, the nursing team will confirm:

- You're awake, alert, and oriented
- Pain is controlled
- Nausea is managed
- You can keep fluids down
- Vital signs are stable
- You have a responsible adult to drive you home and stay with you
- You've urinated (sometimes required, depends on procedure)

Dr. Oluwole's Tip

Your caregiver is just as important as your surgeon. Make sure the person taking you home understands your discharge instructions, knows your medication schedule, and has the surgeon's after-hours number. They are your safety net for the first 24 hours.

12

Nausea & Vomiting

Bonus Module — Prevention and treatment strategies

Post-operative nausea and vomiting (PONV) affects 30–50% of surgical patients and is one of the most common complaints after surgery — often rated by patients as worse than pain. The good news: there are excellent prevention strategies.

Risk Factors for PONV

You are at higher risk if you have:

- Female sex (2–3x higher risk)
- History of PONV or motion sickness
- Non-smoking status
- Younger age
- Use of opioids after surgery
- Certain surgery types (gynecologic, abdominal, ear, eye, laparoscopic)
- Longer anesthesia time

Dr. Oluwole's Tip

Tell your anesthesiologist if you've had nausea after surgery before, if you get motion sickness, or if you're generally prone to nausea. We have a multi-drug prevention protocol that we can tailor specifically to your risk level. The more risk factors you have, the more anti-nausea medications we give during surgery.

Prevention Strategies

During surgery (your anesthesiologist will manage these):

- Ondansetron (Zofran) — most common anti-nausea medication
- Dexamethasone — steroid with anti-nausea properties
- Scopolamine patch (behind the ear) — for high-risk patients

- Minimizing inhaled anesthetics when possible
- Adequate IV hydration

At Home

- **Eat small, bland meals:** Crackers, toast, rice, bananas, applesauce
- **Stay hydrated:** Small sips frequently rather than large amounts at once
- **Ginger:** Ginger tea, ginger chews, or ginger ale (real ginger, not just flavoring) — evidence supports its anti-nausea effect
- **Peppermint:** Peppermint tea or peppermint aromatherapy (inhaling peppermint oil)
- **Fresh air and cool compresses:** Open a window, use a cool cloth on forehead
- **Avoid strong smells:** Cooking odors, perfumes, cleaning products
- **Acupressure:** Press the P6 point (inside of wrist, 3 finger-widths below wrist crease) for 2–3 minutes. Sea-Band wristbands use this principle.

When to Call Your Doctor

- Can't keep any fluids down for 12+ hours
- Can't take your medications due to vomiting
- Signs of dehydration (dark urine, dizziness, dry mouth)
- Vomiting blood or dark "coffee ground" material

13

Constipation After Surgery

Bonus Module — Why it happens and how to prevent it

Post-operative constipation is one of the most underestimated and uncomfortable recovery challenges. Nearly every surgical patient experiences it to some degree. Let's talk about why it happens and how to stay ahead of it.

Why It Happens

- **Opioid medications:** The #1 cause. Opioids slow your entire digestive system.
- **Anesthesia:** General anesthesia temporarily slows gut motility
- **Reduced activity:** Moving less = slower digestion
- **Dehydration:** Not drinking enough water post-op
- **Dietary changes:** Eating less, eating differently, or NPO (nothing by mouth) period
- **Iron supplements:** If prescribed, iron is notoriously constipating

Prevention Protocol

Start BEFORE surgery, not after you're already miserable:

- **Docusate sodium (Colace):** 100mg twice daily — this is a stool softener, not a stimulant. Start the day before surgery. *Must-have if taking opioids.*
- **Senna (Senokot):** 1–2 tablets at bedtime — this is a gentle stimulant laxative. Add if docusate alone isn't enough by day 2 post-op.
- **Miralax (polyethylene glycol):** 17g (1 capful) in 8 oz water once daily — osmotic laxative, safe for daily use

Dr. Oluwole's Tip

My standard advice: if you are prescribed an opioid, you should also be taking a stool softener. Full stop. Don't wait until you're uncomfortable — start it with your first dose of pain medication. Prevention is so much easier than treatment.

High-Fiber Foods to Add When Eating Normally

- Prunes or prune juice (the classic for a reason)
- Pears, apples (with skin), berries
- Oatmeal
- Beans and lentils
- Broccoli, spinach, sweet potatoes
- Whole grain bread
- Ground flaxseed (1–2 tablespoons in yogurt or smoothie)

Hydration

Aim for at least **64 ounces (8 cups) of water daily**. Warm liquids (herbal tea, warm water with lemon) can be especially helpful for stimulating bowel activity.

When to Escalate

- No bowel movement for **3–4 days** despite stool softeners and laxatives → call your surgeon's office
- Severe abdominal pain, bloating, or distension → call your surgeon
- Nausea/vomiting with inability to pass gas → could indicate bowel obstruction → seek urgent care

14

Caregiver's Guide

Bonus Module — For the person helping you recover

To the caregiver: Thank you. You are an essential part of this recovery, and your role matters more than you might realize. This module is specifically for you.

Before Surgery: Home Preparation

☑ Home Prep Checklist (For Caregivers)

- [ ] Move frequently used items to counter/waist height (no reaching or bending)
- [ ] Clear pathways of tripping hazards (rugs, cords, clutter)
- [ ] Set up a recovery station: bed or recliner with pillows, blankets, water, phone charger, remote, medications within reach
- [ ] Stock easy-to-prepare foods: soups, crackers, applesauce, protein shakes, yogurt
- [ ] Fill prescriptions BEFORE surgery day (pharmacy may be closed or busy after)
- [ ] Install grab bars or shower chair if needed (especially for orthopedic patients)
- [ ] Arrange childcare, pet care, or other household responsibilities for recovery period
- [ ] Have the surgeon's office number AND after-hours number saved in your phone

At the Hospital

- **Bring a notebook** — write down everything the surgical team tells you. The patient will NOT remember post-anesthesia instructions.
- **Ask questions** — when can they eat? shower? drive? lift? What medications, and when?
- **Take a photo** of the discharge instructions
- **Bring the car around** when the patient is ready — a wheelchair will take them to the entrance
- **Drive carefully** — every bump will be felt. Bring a pillow for between the seatbelt and incision.

Medication Management

For the first 24–48 hours, YOU should manage the medications:

- Set alarms on your phone for each medication
- Use a medication log: write down what was given and when
- Never let the patient double-dose — anesthesia causes memory fog
- Keep opioids in YOUR possession (the patient should not self-manage opioids while groggy)
- Watch for signs of over-sedation: difficulty waking, very slow breathing

Wound Care

- Wash your hands before and after helping with dressing changes
- Know what the incision looks like now (take a photo at discharge as a baseline)
- Watch for changes: increased redness, swelling, drainage, warmth, or odor
- Help the patient follow the surgeon's wound care instructions exactly

Emotional Support

- Post-surgical depression and mood swings are common — be patient

- The patient may be frustrated with their limitations — validate their feelings
- Encourage small goals: "Let's walk to the kitchen today"
- Limit visitors if the patient is overwhelmed
- Watch for concerning signs: persistent hopelessness, withdrawal, not eating, mentioning self-harm

Self-Care for Caregivers

Dr. Oluwole's Tip

Caregiver burnout is real. You cannot pour from an empty cup. Take breaks, accept help from others, eat well, and sleep when you can. Your well-being matters too — and a rested, healthy caregiver provides better care.

- Accept help when offered — let others bring meals, do laundry, or sit with the patient so you can rest
- Take 15–30 minutes daily for yourself (walk, read, shower in peace)
- Don't neglect your own medications or appointments
- It's okay to feel frustrated, scared, or overwhelmed — you're human

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Understanding & Managing Fatigue After Surgery

Bonus Module — Why you're so tired and what actually helps

Postoperative fatigue affects most surgical patients. After major surgery, about one-third of patients experience significant fatigue lasting throughout the first month. Understanding why it happens and what to do about it helps you recover more smoothly.

How Long Does It Last?

Surgery Type	Typical Fatigue Duration
Minor / day surgery	Mild by day 5
Major surgery	2–4 weeks
Very major (heart, cancer, transplant)	Several weeks to months

Why Does Fatigue Happen?

- **Healing response:** Surgery triggers inflammation; your immune system uses significant energy repairing tissues
- **Anesthesia:** Medications take 24–72 hours to fully clear; sleep patterns are disrupted
- **Pain & opioids:** Pain itself is exhausting, and opioid medications cause drowsiness
- **Deconditioning:** Even a few days of bed rest causes muscle weakness and reduced cardiovascular efficiency
- **Poor nutrition & dehydration:** Reduced appetite + increased calorie needs for healing
- **Emotional stress:** Anxiety, frustration with limitations, worry about recovery

The 6 Best Strategies to Fight Fatigue

1. **Walk daily.** Start short, increase gradually. Activity gives you *more* energy over time, even though it feels tiring at first.

2. **Prioritize sleep.** Keep a regular schedule, limit naps to 20–30 minutes, avoid screens 1 hour before bed, take pain medication as needed to sleep comfortably.
3. **Eat protein at every meal.** Small, frequent meals if appetite is poor. Drink at least 8 glasses of water daily. Limit caffeine after noon.
4. **Manage pain proactively.** Take medications on schedule (not just when pain is severe). Transition off opioids as soon as appropriate — they worsen fatigue.
5. **Pace yourself.** Plan activities when you have the most energy. Alternate activity with rest. Accept help.
6. **Get natural light.** Open curtains, sit by windows, spend time outdoors. Natural light regulates your sleep-wake cycle.

What to Expect Week by Week

Timeframe	What to Expect
Week 1	Fatigue at its worst. Sleep more than usual. Focus on rest + short walks.
Weeks 2–3	Gradual improvement. Can do more each day. May have good and bad days.
Weeks 4+	Steady improvement. Resuming normal activities. Full energy may take months.

When to Be Concerned

Contact your doctor if fatigue is **getting worse** instead of better, prevents basic activities, comes with fever/shortness of breath/chest pain, includes signs of depression, or shows no improvement after several weeks.

Dr. Oluwole's Tip

Fatigue after surgery is normal and expected — and it will improve with time. Staying active within your limits helps more than complete rest. Be patient with yourself, and always contact your doctor if fatigue is severe or not improving.

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Returning to Normal Activities After Surgery

Bonus Module — Driving, work, exercise, travel & more

"When can I get back to normal?" This depends on your surgery type, overall health, and how well you're healing. These are general timelines — always follow your surgeon's specific instructions.

Dr. Oluwole's Tip

Recovery is gradual. Listen to your body — pain is a signal to slow down. When in doubt, ask your surgical team before resuming any activity.

Driving

Do NOT drive if you are taking opioids, unable to brake firmly, or unable to check blind spots.

Surgery Type	Typical Return to Driving
Laparoscopic / minor	1–2 weeks
Open abdominal / C-section	2–6 weeks
Total knee/hip (right side)	1–4 weeks
Total knee/hip (left side, auto trans.)	1–2 weeks
ACL reconstruction	3–6 weeks
Spine fusion	2–6 weeks
Rotator cuff / shoulder	2–3 months
Carpal tunnel release	1–2 weeks

☑ Before You Drive Checklist

- [ ] I can sit comfortably and wear a seatbelt without pain
- [ ] I can turn my head/body to check blind spots
- [ ] I can press the brake pedal firmly and react quickly
- [ ] I am NOT taking opioid pain medications

Returning to Work

Job Type	Minor Surgery	Moderate	Major
Desk / sedentary	1–2 weeks	2–4 weeks	4–6+ weeks
Light physical	2–3 weeks	4–6 weeks	6–8+ weeks
Heavy / manual labor	4–6 weeks	6–12 weeks	3–6+ months

Consider a gradual return (part-time initially). Get a note from your doctor specifying restrictions. Communicate with your employer about limitations.

Exercise Progression

Phase	Timeline	Activities
1	Days–2 weeks	Walking (start short, increase gradually). Gentle movement only.
2	Weeks 2–6	Longer walks, gentle stretching, light daily activities. No lifting/twisting.
3	Weeks 6–12	Low-impact exercise (swimming, stationary bike). Light resistance training.
4	3+ months	Gradual return to full activity. High-impact/contact sports last.

Stop exercising immediately if you experience increased pain, worsening swelling, shortness of breath, chest pain, dizziness, or wound drainage.

Lifting Restrictions

Restriction Level	Max Weight	Reference
Light	5–10 lbs	A gallon of milk (~8 lbs)
Moderate	10–15 lbs	A bag of groceries
Standard	15–20 lbs	A laundry basket

Ask for help. Use a cart for groceries. Have children climb to you instead of lifting them.

Other Activities at a Glance

Activity	General Timeline
Showering	24–48 hours (pat incisions dry)
Bathing / soaking	2–4 weeks (after incisions fully healed)
Swimming	2–4+ weeks (chlorinated pools safer than lakes)
Sexual activity	1–6 weeks depending on surgery (off opioids first)
Light housework	Usually OK early (cooking, dishes, dusting)
Heavy housework	Wait until cleared (vacuuming, mopping, yard work)
Air travel	1–2+ weeks (wear compression stockings, walk aisle, hydrate)
International travel	4–6+ weeks after major surgery

Questions to Ask Your Surgeon

☑ Ask at Your Follow-Up Appointment

- [ ] When can I drive?
- [ ] When can I return to work? What restrictions?
- [ ] What are my lifting restrictions and for how long?
- [ ] When can I exercise, shower, bathe, swim, and travel?
- [ ] When can I resume sexual activity?
- [ ] What signs should prompt me to call you?

Dr. Oluwole's Tip

Your surgeon’s specific instructions override these general guidelines. A gradual return to activities is always safer than rushing. Celebrate your progress, even the small steps.

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Preparing for International Medical Tourism

Bonus Module — What you need to know before surgery abroad

Medical tourism — traveling to another country for medical or surgical care — is chosen for many reasons: lower costs, shorter wait times, access to specific procedures, or combining treatment with travel. While it can offer real benefits, it also carries unique risks that require careful preparation.

Important Risks to Consider

Infection Risks

Infection is the most common complication among medical tourists:

- Surgical site infections
- Blood-borne infections (hepatitis B, hepatitis C, HIV)
- Antimicrobial-resistant infections that may be more common in certain countries
- Infections from contaminated medications or equipment

Several outbreaks have been documented among medical tourists, including fungal meningitis from epidural anesthesia and surgical site infections from nontuberculous mycobacteria after cosmetic surgery.

Other Medical Complications

- Blood clots (deep vein thrombosis, pulmonary embolism)
- Wound healing problems
- Poor surgical outcomes
- Complications from anesthesia
- Death (rare but possible)

Travel-Related Risks

- Air travel after surgery increases blood clot risk
- Changes in cabin pressure can affect healing
- Long flights while in a hypercoagulable state are dangerous
- Limited access to follow-up care if complications occur

Timing Restrictions for Flying

- After chest or abdominal surgery: Wait at least 10 days before flying
- After facial, eyelid, or nose procedures: Wait 7–10 days
- After any major surgery: Consult your surgeon about safe timing

Before You Decide: Research Thoroughly

Evaluate the Facility

- Look for internationally accredited facilities (Joint Commission International, Accreditation Association for Ambulatory Health Care)
- Accreditation does not guarantee a good outcome, but it indicates certain standards are met
- Be aware that marketing websites may not provide complete information about accreditations or qualifications
- Local standards for facility accreditation may differ from those in the United States

Evaluate the Healthcare Professional

- Verify the surgeon is certified in their specialty through a process equivalent to American Board of Medical Specialties certification
- Organizations that accredit physicians abroad include the American College of Surgeons, American Society of Plastic Surgeons, and International Society of Aesthetic Plastic Surgery
- Request outcome data if available (though this may not be tracked in all locations)
- Ask about the surgeon's experience with your specific procedure

Understand What's Included

- Clarify exactly what services are included in the quoted cost
- Ask about fees for follow-up care, complications, or additional procedures
- Determine where follow-up care will occur
- Understand that pharmaceuticals, laboratory services, and medical devices may not be subject to the same regulatory oversight as in the United States

Essential Pre-Travel Steps

☑ 4–6 Weeks Before Travel

- [ ] Discuss your plans with your primary care physician in the United States
- [ ] Consider consulting a travel medicine specialist
- [ ] Ensure all routine vaccinations are up to date, especially hepatitis B
- [ ] Review current medications and ensure adequate supply for the entire trip plus extra
- [ ] Discuss how to manage any chronic conditions during travel

Arrange for Complications

- Discuss the plan for addressing complications with both your U.S. healthcare professional and the overseas provider
- Identify healthcare facilities at your destination that can handle emergencies
- Know how to contact the U.S. Embassy or Consulate
- Enroll in the U.S. Department of State's Smart Traveler Enrollment Program (STEP)

Insurance Considerations

- Check your domestic health insurance carefully — most do not cover care received abroad
- Purchase supplemental medical insurance that covers international care
- Obtain medical evacuation insurance (essential for remote destinations)
- Understand that if complications develop, you may not have the same legal recourse as in the United States

☑ Documentation to Prepare

- [ ] Letter from your healthcare professional listing all medical conditions
- [ ] Current medications list (with generic names and doses)
- [ ] Allergy documentation
- [ ] Information about any required medical equipment
- [ ] Download travel health apps to store medical records
- [ ] Carry copies of prescriptions
- [ ] Consider medical identification jewelry if you have significant health conditions

During Your Trip

Before the Procedure

- Meet with your surgeon and anesthesiologist
- Ask all your questions and ensure you understand the procedure
- Confirm the plan for postoperative care
- Know the signs of complications and when to seek help

After the Procedure

- Follow all postoperative instructions carefully
- Avoid typical vacation activities during recovery:
  - No alcohol consumption
  - No strenuous activity or exercise
  - No sunbathing or swimming
  - No long tours or sightseeing
- Do not delay seeking medical care if you suspect any complication

- Request complete copies of your medical records in English before leaving

☑ Before Flying Home

- [ ] Confirm you are medically cleared to fly
- [ ] Understand the risks of flying too soon after surgery
- [ ] Plan to walk the aisle regularly during the flight
- [ ] Pack compression stockings if recommended
- [ ] Have blood clot prevention medication if prescribed
- [ ] Carry all medical records from your overseas care

After Returning Home

Immediate Steps

- Schedule a follow-up appointment with a U.S. healthcare professional
- Provide complete information about your travel history, surgeries received, medications given, and any complications
- Share all medical records from your overseas care

Watch for Complications

Seek medical care immediately if you experience:

- Fever or chills
- Increasing pain, redness, or swelling at the surgical site
- Wound drainage or opening
- Shortness of breath or chest pain
- Leg swelling, warmth, or pain (possible blood clot)
- Any other concerning symptoms

Dr. Oluwole's Tip

Inform any healthcare professionals you see about your overseas medical care, even if it occurred up to 12 months ago. Some patients have returned with antimicrobial-resistant infections that require special treatment. Complete disclosure helps ensure proper care.

Questions to Ask Before Committing

About the Facility

- Is the facility accredited by an international accrediting organization?
- What are the facility's infection rates and complication rates?
- What emergency services are available on-site?
- How are medical records maintained and secured?

About the Surgeon

- What are the surgeon's credentials and certifications?
- How many of these procedures has the surgeon performed?
- What are the surgeon's complication rates?
- Can I speak with previous patients?

About the Procedure

- What exactly is included in the cost?
- What happens if I have a complication?
- Who will provide follow-up care?
- When will I be cleared to fly home?
- How will my care be coordinated with my U.S. healthcare team?

About Legal Protections

- What legal recourse do I have if something goes wrong?
- Is there malpractice insurance coverage?
- What are my rights as a patient in this country?

Dr. Oluwole's Tip

Medical tourism must be voluntary — never feel pressured. Accreditation does not guarantee a good outcome. Lower cost does not mean lower quality, but it also doesn't guarantee equivalent care. Having a plan for follow-up care in the United States is essential. Your safety should always be the top priority.

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Preparing for Gynecologic Surgery

Bonus Module — What every patient should know

Gynecologic surgery includes procedures on the female reproductive organs — the uterus, ovaries, fallopian tubes, cervix, vagina, and vulva. These surgeries may be performed for many reasons, including fibroids, endometriosis, ovarian cysts, abnormal bleeding, pelvic organ prolapse, or cancer.

Types of Gynecologic Surgery

- **Hysterectomy:** Removal of the uterus (may include ovaries and fallopian tubes)
- **Myomectomy:** Removal of uterine fibroids
- **Oophorectomy:** Removal of one or both ovaries
- **Salpingectomy:** Removal of fallopian tubes
- **Procedures for prolapse:** Repairs for pelvic organ prolapse
- **Endometriosis surgery:** Removal of endometriosis tissue
- **Diagnostic procedures:** Laparoscopy, hysteroscopy

Surgical Approaches

- **Vaginal surgery:** Through the vagina with no abdominal incisions
- **Laparoscopic surgery:** Small incisions with a camera and instruments
- **Robotic surgery:** Laparoscopic surgery assisted by robotic technology
- **Open (abdominal) surgery:** Larger incision in the abdomen

Minimally invasive approaches (vaginal, laparoscopic, robotic) generally have faster recovery times and fewer complications when appropriate for your condition.

Before Your Surgery

Preoperative Appointments

You will have appointments to:

- Discuss the procedure, risks, benefits, and alternatives
- Review your medical history and current medications
- Complete any necessary tests (blood work, imaging, EKG)
- Meet with anesthesia if needed
- Receive instructions for surgery day

Medications to Discuss

Tell your surgical team about all medications, including:

- Blood thinners (may need to be stopped before surgery)
- Diabetes medications (may need adjustment)
- Hormone therapy and birth control pills
- Herbal supplements (some increase bleeding risk)
- Over-the-counter medications

☑ Preparing Your Body

- [ ] Shower or bathe the night before surgery with soap (antimicrobial or regular)
- [ ] Do NOT shave the surgical area yourself — shaving with a razor increases infection risk
- [ ] Treat any infections before surgery (UTIs, skin infections, etc.)

Fasting Instructions

Modern guidelines allow:

- Clear liquids (water, apple juice, black coffee, tea without milk) up to 2 hours before surgery
- Light solid foods up to 6 hours before surgery
- Heavy or fatty meals should be avoided for at least 8 hours

Follow your specific instructions — they may vary based on your procedure.

Bowel Preparation

For most gynecologic surgeries, especially minimally invasive procedures, bowel preparation is **NOT routinely needed**. Your surgeon will tell you if bowel prep is required for your specific procedure. The old practice of routine bowel prep has been abandoned for most gynecologic surgeries.

The Day of Surgery

- Arrive at the time specified (usually 1–2 hours before surgery)
- You will change into a hospital gown
- An IV will be placed for fluids and medications
- You will meet your surgical and anesthesia team
- The surgical site will be cleaned with antiseptic solution
- You will receive antibiotics before surgery to prevent infection

Preventing Blood Clots

- You may receive compression stockings or inflatable leg devices
- Early walking after surgery is encouraged
- Blood-thinning medication may be given depending on your risk factors

After Your Surgery

Modern Enhanced Recovery After Surgery (ERAS) protocols help you recover faster:

- **Early eating and drinking:** You can usually eat and drink soon after surgery
- **Early walking:** Getting up and walking as soon as possible helps prevent complications
- **Pain management:** A combination of medications (not just opioids) controls pain better with fewer side effects
- **Early removal of catheter:** If you have a urinary catheter, it will be removed as soon as possible

Pain Management

Your pain will be managed with multiple types of medications:

- Non-opioid pain relievers (acetaminophen, ibuprofen) are the foundation
- Opioid medications are available for breakthrough pain
- This “multimodal” approach provides better pain control with fewer side effects

☑ Before Discharge — Make Sure You Understand

- [ ] How to care for your incisions
- [ ] What medications to take and when
- [ ] Activity restrictions
- [ ] When to call your doctor
- [ ] Your follow-up appointment date

Recovery at Home

Activity Guidelines

- **Walking:** Start walking right away and increase gradually
- **Lifting:** Avoid heavy lifting (usually >10–15 pounds) for 4–6 weeks
- **Driving:** Usually 1–2 weeks after laparoscopic surgery, longer after open surgery
- **Work:** Depends on your job and type of surgery
- **Exercise:** Gradual return as directed by your surgeon
- **Sexual activity:** Usually wait 6 weeks after hysterectomy; ask your surgeon for other procedures

Wound Care

- Keep incisions clean and dry initially
- You can usually shower 24–48 hours after surgery
- No soaking in tubs, pools, or hot tubs until incisions are fully healed

- Watch for signs of infection (redness, swelling, drainage, fever)

What to Expect

- Some vaginal bleeding or discharge is normal after many gynecologic procedures
- Fatigue is common and may last several weeks
- Constipation is common — use stool softeners as recommended
- Emotional changes may occur, especially after hysterectomy or removal of ovaries

When to Call Your Doctor

Contact your surgical team if you experience:

- Fever (temperature over 100.4°F or 38°C)
- Heavy vaginal bleeding (soaking more than one pad per hour)
- Increasing abdominal pain
- Redness, swelling, or drainage from incisions
- Difficulty urinating or inability to urinate
- Nausea or vomiting that won't stop
- Leg swelling, warmth, or pain
- Shortness of breath or chest pain

Special Considerations

If You're Taking Hormones

- Combined oral contraceptives and hormone therapy slightly increase blood clot risk
- However, stopping them before surgery is not routinely recommended
- If you have additional risk factors for blood clots, discuss this with your surgeon
- Blood clot prevention measures will be used during and after surgery

If You Have Diabetes

- Blood sugar control is important for healing and preventing infection
- Your diabetes medications may need adjustment before and after surgery
- Blood sugar will be monitored during your hospital stay

If You're Overweight

- Antibiotic doses may be adjusted for your weight
- Blood clot prevention is especially important
- Minimally invasive surgery may still be possible

Dr. Oluwole's Tip

Follow all preoperative instructions carefully. Don't shave the surgical area yourself. You can eat and drink closer to surgery than you might expect. Early walking and eating after surgery help you recover faster. Pain can be well controlled with a combination of medications — and most complications are preventable with proper preparation and care.

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Cesarean Section Recovery

Bonus Module — What to expect and how to heal

A cesarean section (C-section) is a surgical procedure to deliver your baby through an incision in your abdomen and uterus. Whether your cesarean was planned or unplanned, understanding what to expect during recovery will help you heal well while caring for your newborn.

Immediately After Surgery

In the Recovery Room

- You will be monitored closely as anesthesia wears off
- If you had spinal or epidural anesthesia, your legs will be numb for a few hours
- You may feel shaky or cold — this is normal
- You can usually hold and breastfeed your baby soon after surgery
- Pain medication will be given to keep you comfortable

The First 24 Hours

- Your IV will provide fluids until you can drink well
- You can start drinking clear liquids soon after surgery
- You can eat a regular diet within 2 hours of surgery (unless your doctor advises otherwise)
- Your urinary catheter will be removed as soon as possible (often within 6–12 hours)
- You will be encouraged to get up and walk within hours of surgery

Pain Management

Multimodal Approach

Your pain will be managed with a combination of medications that work better together:

- **Scheduled acetaminophen (Tylenol):** Given regularly, not just when you have pain
- **Scheduled NSAIDs (ibuprofen or ketorolac):** Anti-inflammatory medications that are very effective for post-cesarean pain and safe while breastfeeding
- **Opioids for breakthrough pain:** Available if needed, but the goal is to minimize opioid use

What to Expect

- Some discomfort is normal, but severe pain should be reported
- Pain is usually worst in the first 2–3 days and improves steadily
- Taking pain medication on schedule works better than waiting until pain is severe
- All recommended pain medications are safe while breastfeeding

Dr. Oluwole's Tip

Don't be a hero with pain medication. Taking it on schedule — before pain gets severe — actually means you'll need less medication overall and recover faster. All the recommended medications are safe for breastfeeding.

Your Hospital Stay

Early Mobilization

Getting up and moving is one of the most important things you can do:

- Helps prevent blood clots
- Improves bowel function
- Reduces risk of complications
- Helps you feel better faster

You will be encouraged to sit up in bed within hours of surgery, walk to the bathroom with assistance, walk in the hallway several times a day, and increase activity each day.

Eating and Drinking

- You can drink fluids soon after surgery
- You can eat a regular diet within hours of surgery
- Eating helps your bowels recover
- Stay well hydrated, especially if breastfeeding

Preventing Blood Clots

- Compression stockings or inflatable leg devices
- Early and frequent walking
- Staying hydrated
- Blood-thinning medication if you have additional risk factors

Breastfeeding

- You can breastfeed soon after cesarean delivery
- Pain medications used are safe for breastfeeding
- Nurses and lactation consultants can help with positioning
- Side-lying or football hold positions may be more comfortable
- Skin-to-skin contact with your baby is encouraged

Going Home

You may be ready to go home when your pain is controlled with oral medications, you can eat and drink normally, you can walk and care for yourself, you can urinate normally, and your incision looks healthy. Many women go home 2–3 days after cesarean delivery.

☑ Before Discharge

- [ ] I understand how to care for my incision
- [ ] I know what medications to take and when
- [ ] I know the warning signs that need medical attention
- [ ] I have my follow-up appointment scheduled
- [ ] I know how to get help if I have questions

Recovery at Home

The First Two Weeks

- Rest when your baby sleeps
- Accept help from family and friends
- Walk regularly but avoid strenuous activity
- Don't lift anything heavier than your baby
- Take pain medication as needed
- Eat nutritious foods and stay hydrated
- Continue stool softeners to prevent constipation

Incision Care

- Keep the incision clean and dry
- Shower daily — let water run over the incision
- Pat dry gently
- Wear loose, comfortable clothing
- No soaking in tubs, pools, or hot tubs until fully healed (usually 4–6 weeks)

Activity Guidelines

- **Walking:** Encouraged from day one; increase gradually
- **Stairs:** Use as needed, but take it slow

- **Driving:** Usually wait 2–4 weeks (until you can brake suddenly without pain)
- **Lifting:** Nothing heavier than your baby for 4–6 weeks
- **Exercise:** Gentle walking only initially; ask your doctor about returning to exercise
- **Sexual activity:** Usually wait 6 weeks until your postpartum checkup

Emotional Recovery

- "Baby blues" (mood swings, tearfulness) are common in the first 2 weeks
- Cesarean delivery can bring mixed emotions
- Talk about your feelings with your partner, family, or healthcare provider
- Seek help if you feel persistently sad, anxious, or unable to care for yourself or your baby

Warning Signs: When to Call Your Doctor

Incision Problems

- Increasing redness, swelling, or warmth around the incision
- Drainage from the incision (especially if cloudy or foul-smelling)
- Incision opening up
- Fever (temperature over 100.4°F or 38°C)

Bleeding

- Heavy vaginal bleeding (soaking more than one pad per hour)
- Large blood clots
- Foul-smelling vaginal discharge

Other Concerns

- Severe or worsening abdominal pain
- Pain, redness, or swelling in your legs
- Chest pain or difficulty breathing
- Painful urination or inability to urinate
- Severe headache that doesn't improve with medication

- Signs of depression or thoughts of harming yourself or your baby

Long-Term Recovery

4–6 Weeks

- Most women feel significantly better by this time
- You will have a postpartum checkup
- You can usually resume normal activities, including exercise and sexual activity
- Your incision should be well healed

Full Recovery

- Complete healing takes 6–8 weeks or longer
- Some numbness around the incision is normal and may last months
- The scar will fade over time
- Future pregnancies are usually possible after cesarean delivery

Dr. Oluwole's Tip

Remember: you are recovering from major surgery while caring for a newborn. Be patient with yourself. Accept help. Pain should improve steadily each day. Early walking and eating help you recover faster. Breastfeeding is safe and encouraged. And always seek help if something doesn’t feel right.

20 Pain Management During Labor & Delivery

Bonus Module — Making an informed choice

Labor pain is among the most intense types of physical pain. The good news: there are many effective options for managing it. Understanding your choices will help you make informed decisions and communicate your preferences to your healthcare team.

> **Dr. Oluwole's Tip**
>
> There is no “right” way to manage labor pain. Your preferences may change during labor — and that’s okay. Discuss options with your provider before labor begins, but keep an open mind.

Epidural Analgesia

The most common method of pain relief during labor, used by more than 60% of women giving birth in the United States.

How It Works

- A thin catheter is placed in the epidural space near your spinal cord
- Local anesthetic and pain medication are delivered continuously
- Pain relief begins within 10–20 minutes
- You remain awake and alert
- Can be adjusted throughout labor

Important Facts

- Epidurals do **NOT** increase the risk of cesarean delivery
- Modern epidurals do **NOT** increase the risk of assisted vaginal delivery
- Epidurals do **NOT** cause long-term back pain
- Epidurals are safe for your baby
- Can be used for cesarean delivery if needed

Possible Side Effects

- Temporary drop in blood pressure (treated with fluids or medication)
- Itching (treatable)
- Fever (about 30% of women; usually not from infection)
- Headache (rare, less than 1% of women)
- Your legs may feel heavy or tingly

Combined Spinal-Epidural (CSE)

Combines rapid onset of spinal anesthesia with the ongoing relief of an epidural catheter. Popular choice that provides the benefits of both techniques.

IV Opioid Medications

Opioid pain medications given through your IV provide moderate pain relief.

- Common medications: fentanyl, morphine, remifentanil
- Available in almost all birth settings
- Allow you to remain mobile
- Less effective than epidural for pain relief
- May cause drowsiness or nausea
- Usually avoided close to delivery to minimize effects on baby

Nitrous Oxide (Laughing Gas)

- You control when and how much you breathe through a mask
- Provides mild to moderate pain relief and reduces anxiety
- Wears off quickly between contractions
- Safe for your baby
- Can be used while waiting for an epidural
- Not available at all hospitals

Non-Medication Methods

Many women use these alone or in combination with medications:

- **Movement:** Walking, birthing ball, position changes, rocking
- **Water therapy:** Warm shower, laboring in a tub, warm compresses
- **Breathing & relaxation:** Focused breathing, meditation, visualization
- **Physical comfort:** Massage, counterpressure, heat/cold packs
- **Support:** Continuous labor support from partner, doula, or nurse
- **Other:** Acupuncture, TENS, hypnosis (requires prior preparation)

Comparing Your Options

Method	Pain Relief	Mobility	Availability
Epidural/Spinal	Most effective	Limited	Most hospitals
IV Opioids	Moderate	May cause drowsiness	Almost universal
Nitrous Oxide	Mild-moderate	Full	Some hospitals
Non-medication	Varies	Full	Universal

Dr. Oluwole's Tip

There is no “wrong” choice for pain management. The best choice is what works for you. You can combine different methods, and your preferences may change during labor. Trust your healthcare team to help you make the best decision in the moment.

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Nutrition for Surgical Recovery

Bonus Module — Eating to heal

Your body needs extra nutrients to heal after surgery. Good nutrition helps repair tissues, fight infection, rebuild strength, restore energy, and reduce complications. Eating well after surgery is one of the most important things you can do.

Key Nutrients for Healing

Protein: The Building Block of Healing

- Most surgical patients need **1.2–1.5 grams per kg** of body weight daily
- For a 150-pound person, that's about **80–100 grams of protein per day**
- Good sources: meat, poultry, fish, eggs, dairy, beans, lentils, nuts, tofu
- Aim for protein at every meal and snack

Vitamins & Minerals

- **Vitamin C:** Essential for collagen and wound healing — citrus, berries, peppers, broccoli
- **Vitamin A:** Supports immune function and tissue repair — orange/yellow vegetables, leafy greens, eggs
- **Zinc:** Critical for wound healing and immunity — meat, shellfish, legumes, nuts, seeds
- **Iron:** Needed for oxygen delivery to healing tissues — red meat, poultry, fish, beans, fortified cereals

Fluids

- Drink at least 8 glasses of water daily
- More if you have drains, fever, or are losing fluids
- Water, milk, and diluted juices are good choices
- Limit caffeine and avoid alcohol during recovery

Eating After Surgery: A Timeline

The First Few Days

- Start with clear liquids as directed, progress to regular foods as tolerated
- Eat small, frequent meals
- Focus on easy-to-digest foods

The First Week

- Gradually increase portion sizes
- Include protein at every meal
- Eat a variety of fruits and vegetables
- Take any prescribed supplements

Weeks 2–4 and Beyond

- Return to a normal, balanced diet
- Continue emphasizing protein
- Maintain good hydration

Tips for Common Recovery Challenges

If Your Appetite Is Poor

- Eat small, frequent meals (5–6 times per day)
- Choose nutrient-dense foods over empty calories
- Add protein powder to smoothies, soups, or oatmeal
- Drink nutritional supplements (Ensure, Boost) between meals

If You Have Nausea

- Eat bland, dry foods (crackers, toast, rice)
- Avoid greasy, spicy, or strong-smelling foods
- Sip clear liquids between meals
- Try ginger tea or ginger candies

If You Have Constipation

- Drink plenty of fluids
- Eat high-fiber foods (fruits, vegetables, whole grains)
- Prunes and prune juice are natural laxatives
- Take stool softeners as recommended

Foods to Emphasize

- **High-protein:** Chicken, fish, eggs, Greek yogurt, cottage cheese, beans, tofu
- **Healing-supportive:** Colorful fruits and vegetables, whole grains, healthy fats (olive oil, avocado), bone broth, fermented foods (yogurt, kefir)

Foods to Limit

- Alcohol (interferes with healing and medications)
- Excessive sugar (empty calories, impairs immune function)
- Highly processed foods (low in nutrients)
- Very salty foods (can cause fluid retention)

Sample Meal Ideas

Meal	Options
Breakfast	Scrambled eggs with cheese & toast • Greek yogurt with berries • Oatmeal with nuts • Protein smoothie
Lunch	Chicken salad sandwich • Lentil soup with crackers • Tuna salad • Bean and cheese quesadilla
Dinner	Baked salmon with vegetables • Chicken stir-fry • Beef stew • Pasta with meat sauce
Snacks	Cheese & crackers • Nuts • Yogurt • Hard-boiled eggs • PB on apple slices

BARIATRIC SURGERY NOTE

Follow your bariatric team's specific diet progression (see Module 6). Protein is especially important. Take all recommended vitamins and supplements for life. Eat slowly and chew thoroughly.

Dr. Oluwole's Tip

Good nutrition is one of the most powerful tools for healing. If you're struggling to eat enough, don't be afraid to use nutritional supplements like Ensure or Boost between meals. And contact your healthcare team if you can't eat or drink for more than 24 hours.

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Preparing for Cancer Surgery

Bonus Module — A complete patient guide

Surgery is one of the most important treatments for cancer. It may be used to remove a tumor, determine the extent of disease, relieve symptoms, or reconstruct areas affected by cancer. Preparing well can reduce complications, speed recovery, and improve your overall outcome.

Your Cancer Surgery Team

Cancer surgery involves a multidisciplinary team working together:

- **Surgical oncologist:** Specializes in cancer operations
- **Medical oncologist:** Manages chemotherapy and systemic treatments
- **Radiation oncologist:** Manages radiation therapy
- **Anesthesiologist:** Manages anesthesia and pain during surgery
- **Pathologist:** Examines tissue removed during surgery
- **Dietitian, physical therapist, social worker, psychologist:** Support your preparation and recovery

It is important that all team members evaluate your case before treatment begins.

Prehabilitation for Cancer Surgery

Just as athletes train before a competition, you can train your body before surgery. Studies show prehabilitation can **reduce postoperative complications by up to 40%**.

Exercise

- Aim for at least 30 minutes of moderate activity most days (walking, cycling, swimming)
- Even small amounts are better than none
- Your team can design a program tailored to your abilities

Nutrition

- Eat plenty of protein to maintain muscle mass
- If you've lost weight or have a poor appetite, tell your team
- Immunonutrition formulas (arginine, omega-3 fatty acids) may be recommended before major abdominal cancer surgery

Mental & Emotional Preparation

- Distress is highest around diagnosis and surgery — and gradually improves
- Deep breathing, meditation, guided imagery, and support groups all help
- Your cancer center should screen you for distress and connect you with resources

> **Dr. Oluwole's Tip**
>
> Ideally, prehabilitation should begin as soon as surgery is planned. If you're receiving chemotherapy or radiation before surgery, you may have weeks or months to prepare. Use this time wisely.

Quitting Smoking Before Cancer Surgery

If you smoke, quitting before surgery is one of the most important things you can do:

- Smoking increases risk of lung complications, wound infections, blood clots, and longer hospital stays
- Even quitting on the day of surgery has some benefit
- Four weeks of abstinence significantly reduces complications
- Nicotine replacement therapy (patches, gum) is safe before surgery and is NOT a reason to delay
- Quitting should never delay your cancer surgery

Medical Optimization

- **Diabetes:** Blood sugar control is critical for healing and preventing infection
- **Heart/lung disease:** May need evaluation before surgery

- **Anemia:** Low blood counts may need correction before surgery
- **Dental care:** Important before head/neck cancer surgery or radiation

Understanding Your Surgery

Surgical Approaches

- **Open surgery:** Larger incision for direct access
- **Minimally invasive (laparoscopic/robotic):** Smaller incisions, often faster recovery
- **Endoscopic:** Through natural openings

Lymph Node Surgery

- **Sentinel node biopsy:** Removing only the first node(s) where cancer would spread
- **Lymph node dissection:** Removing a group of lymph nodes
- May cause lymphedema (swelling) — your team will discuss prevention

After Cancer Surgery

Enhanced Recovery (ERAS)

- Early eating and drinking
- Early walking to prevent complications
- Multimodal pain management (not just opioids)
- Early removal of tubes and drains
- Blood clot prevention

ERAS protocols reduce hospital stays and lower complication rates across many types of cancer surgery.

Drains and Tubes

You may go home with surgical drains. Your team will teach you how to empty them, measure output, and recognize problems.

Pathology Results

After surgery, tissue is examined by a pathologist (takes several days to a week+). The report determines whether all cancer was removed, the exact type and grade, lymph node involvement, and what additional treatment may be needed.

Special Considerations by Cancer Type

Breast Cancer

- Lumpectomy or mastectomy; sentinel node biopsy common
- May go home with drains
- PT can prevent shoulder stiffness and lymphedema
- Discuss reconstruction options before surgery

Lung Cancer

- Breathing exercises before surgery are especially important
- Chest tubes after surgery
- Early mobilization is critical
- Pulmonary rehabilitation may be recommended

Colorectal Cancer

- May need a temporary or permanent stoma (ostomy)
- If a stoma is planned, you'll meet with a stoma nurse beforehand
- Early eating and walking are especially important

Head & Neck Cancer

- Dental evaluation before surgery is essential
- Speech and swallowing therapy may be needed
- May need a temporary feeding tube

Preparing for Additional Treatment

Many cancer patients need chemotherapy or radiation after surgery. One of the goals of good surgical recovery is to be well enough to start additional treatment on time. Prehabilitation and ERAS protocols help ensure this.

☑ Cancer Surgery Preparation Checklist

- [ ] I understand my diagnosis and surgical plan
- [ ] I've asked all my questions
- [ ] I've started prehabilitation (exercise, nutrition, stress management)
- [ ] If I smoke, I've started a quit plan
- [ ] Chronic conditions are optimized
- [ ] Medications reviewed with surgical team
- [ ] Preoperative tests completed
- [ ] Home help arranged for recovery
- [ ] Home prepared for recovery
- [ ] Advance directive completed (if desired)

Dr. Oluwole's Tip

Preparing well for cancer surgery genuinely improves your outcome. Prehabilitation, quitting smoking, managing distress, and following ERAS protocols all make a measurable difference. Your cancer team is there to support you every step of the way. Don't hesitate to ask questions or express concerns. **Book an OpWell consultation for personalized surgical preparation →** (https://www.opwellconcierge.com/schedule)

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Your Complete Guide to Labor & Delivery

Bonus Module — Everything expectant mothers need in one place

This module brings together everything you need to prepare for labor, delivery, and the early postpartum period — whether you're planning a vaginal birth or a cesarean section. Think of this as your L&D home base.

> **Dr. Oluwole's Tip**
>
> As an anesthesiologist, I've been in the delivery room thousands of times. The patients who do best are the ones who walk in informed and confident — not because nothing will surprise them, but because they understand the process and trust their team. That's what this guide is for.

Preparing for Delivery Day

When to Go to the Hospital

- **Regular contractions:** 5 minutes apart, lasting 1 minute each, for at least 1 hour (the "5-1-1 rule")
- **Your water breaks** — note the time, color, and odor and call your provider
- **Heavy vaginal bleeding** (more than spotting)
- **Decreased fetal movement** — if your baby is moving less than usual
- **Severe headache, vision changes, or upper abdominal pain** — could be preeclampsia
- **Scheduled C-section:** Arrive at the time your OB team specifies (usually 1–2 hours before)

☑ Hospital Bag Checklist

- [ ] Photo ID and insurance cards
- [ ] Birth plan (printed copies for your nurse and anesthesiologist)
- [ ] Comfortable robe or gown for laboring
- [ ] Nursing bra and breast pads
- [ ] High-waisted underwear (won't rub a C-section incision)
- [ ] Going-home outfit for you (loose, comfortable)
- [ ] Going-home outfit for baby + swaddle blanket
- [ ] Infant car seat installed in the car
- [ ] Phone charger (long cord)
- [ ] Toiletries, lip balm, hair ties
- [ ] Snacks for your support person
- [ ] Pillow from home (optional but comforting)

Understanding the Stages of Labor

Stage	What Happens	How Long
Early labor	Cervix dilates 0–6 cm. Contractions are mild to moderate, 5–30 min apart. You can usually stay home during this phase.	Hours to days (varies widely)
Active labor	Cervix dilates 6–10 cm. Contractions are strong, regular, 2–5 min apart. This is when most women go to the hospital and may request an epidural.	4–8 hours (first baby); shorter for subsequent
Pushing & delivery	Cervix is fully dilated. You push with contractions to deliver your baby.	Minutes to 2–3 hours

Placenta delivery	The placenta is delivered. Uterine massage and any repairs are done.	5–30 minutes

Your Birth Plan

A birth plan communicates your preferences to your care team. Keep it to 1 page and stay flexible — labor is unpredictable.

☑ Birth Plan Decisions to Discuss

- [ ] Pain management preference (epidural, nitrous, unmedicated, open to all options)
- [ ] Who I want in the delivery room (partner, doula, family member)
- [ ] Mobility during labor (walking, birthing ball, tub if available)
- [ ] Immediate skin-to-skin contact after delivery
- [ ] Delayed cord clamping preference
- [ ] Breastfeeding plan (initiate within first hour)
- [ ] If C-section becomes necessary: clear drape? Skin-to-skin in OR?
- [ ] Cord blood banking (if planned, have the kit at the hospital)

Anesthesia & Pain Management Options

Understanding your pain relief options *before* labor gives you the power to make informed decisions in the moment. Here's a quick overview — for the full deep dive, see **Module 20: Pain Management During Labor & Delivery**.

Method	Pain Relief	Key Facts

Epidural	Most effective	Used by 60%+ of US births. Does NOT increase C-section risk. Safe for baby. Can be adjusted throughout labor.
Combined Spinal-Epidural	Most effective, faster onset	Rapid relief (minutes) + ongoing epidural. Popular choice.
IV opioids	Moderate	Available early in labor. May cause drowsiness. Usually avoided near delivery.
Nitrous oxide	Mild-moderate	You control it. Wears off quickly. Not available everywhere.
Non-medication	Varies	Breathing, massage, water therapy, movement, doula support. Can combine with any of the above.

Dr. Oluwole's Tip

There is no "right" choice. The best pain management is whatever works for you. Your preferences may change during labor — and that's completely okay. An epidural at 3 AM after 12 hours of labor is not a failure. It's a tool.

If You're Having a Planned Cesarean

For the full recovery guide, see **Module 19: Cesarean Section Recovery**. Key points:

- You'll likely receive **spinal or epidural anesthesia** — you'll be awake for the birth
- Your support person can usually be in the OR with you
- Skin-to-skin contact can often happen shortly after delivery
- Surgery takes about 45–60 minutes; you'll feel pressure but not pain
- Recovery in hospital is typically 2–3 days
- All recommended pain medications are safe for breastfeeding

If an Unplanned Cesarean Becomes Necessary

About 1 in 3 US deliveries are cesarean. Sometimes a C-section is needed during labor for:

- Baby showing signs of distress (abnormal heart rate patterns)
- Labor not progressing despite adequate contractions
- Umbilical cord issues
- Placental problems

If you have an epidural in place, it can usually be used for the C-section — no general anesthesia needed. If not, a spinal is placed quickly. Your support person can typically stay with you.

Breastfeeding Basics

- **First hour:** Skin-to-skin contact and early breastfeeding are encouraged (even after C-section)
- **Colostrum** (the first milk) is small in volume but packed with antibodies — it's all your baby needs at first
- Milk typically "comes in" 2–5 days after birth
- Breastfeeding can be uncomfortable at first — ask for a **lactation consultant** (most hospitals have them)
- **C-section positions:** Side-lying or football hold may be more comfortable than cradle hold
- All standard postpartum pain medications (acetaminophen, ibuprofen, most opioids) are safe while breastfeeding

Postpartum Recovery

What's Normal

- **Vaginal bleeding (lochia):** Lasts 4–6 weeks. Starts heavy, gradually lightens. Use pads only — no tampons.
- **Cramping:** Uterine contractions as your uterus shrinks back. Worse during breastfeeding (oxytocin). Normal.

- **Perineal soreness** (vaginal delivery): Ice packs, sitz baths, and witch hazel pads help.
- **Incision pain** (C-section): Worst in first 2–3 days. Take pain medication on schedule.
- **Baby blues:** Mood swings, tearfulness, feeling overwhelmed — affects up to 80% of new mothers. Usually resolves within 2 weeks.
- **Fatigue:** Profound. Sleep when baby sleeps. Accept all help offered.
- **Constipation:** Very common. Start stool softeners before leaving the hospital.

When to Call Your Doctor

- Fever above 100.4°F (38°C)
- Heavy bleeding (soaking more than 1 pad per hour) or large clots
- Foul-smelling vaginal discharge
- Incision redness, swelling, opening, or drainage (C-section)
- Severe headache that doesn't improve, vision changes, or upper abdominal pain (possible **postpartum preeclampsia** — can occur up to 6 weeks after delivery)
- Leg swelling, warmth, or pain (possible blood clot)
- Chest pain or difficulty breathing
- Painful, red, warm breast with fever (possible mastitis)
- Difficulty urinating or painful urination

Postpartum Mental Health

"Baby blues" vs. postpartum depression:

Baby Blues (Normal)	Postpartum Depression (Seek Help)
Starts within days of delivery	Can start anytime in the first year
Mood swings, tearfulness, irritability	Persistent sadness, hopelessness, numbness
Resolves within 2 weeks	Lasts more than 2 weeks and worsens

You can still care for yourself and baby	Difficulty bonding, caring for baby, or functioning
Comes and goes	Constant or worsening

IMPORTANT

Postpartum depression is a medical condition, not a character flaw. If you experience persistent sadness, inability to care for yourself or your baby, anxiety that won't stop, or thoughts of harming yourself or your baby — tell someone immediately. Call your OB, go to the ER, or call the Postpartum Support Helpline: **1-800-944-4773**.

OpWell's mental wellness specialist, **Stacey J. Floyd, MA, LPC**, provides perinatal mental health support. **Book a session →** (https://www.opwellconcierge.com/schedule)

Your L&D Preparation Timeline

When	What to Do
28–32 weeks	Take a childbirth class. Tour the hospital/birth center. Start your birth plan. Begin pelvic floor exercises.
32–36 weeks	Pack your hospital bag. Install the car seat. Discuss anesthesia options with your provider. Consider an OpWell L&D Consultation.
36–38 weeks	Finalize birth plan. Pre-register at the hospital. Stock freezer meals. Arrange help for after delivery.
38–40 weeks	Review when to go to the hospital. Have your bag by the door. Rest as much as possible.
After delivery	Focus on rest, feeding, bonding. Take pain meds on schedule. Accept help. Watch for warning signs.

Deep-Dive Modules

This guide covers the big picture. For detailed deep-dives, see:

- **Module 19: Cesarean Section Recovery** — complete C-section recovery guide (pain management, incision care, breastfeeding, emotional recovery, week-by-week timeline) **Open Module 19 →**
- **Module 20: Pain Management During Labor & Delivery** — detailed comparison of all pain relief options (epidural, spinal, CSE, IV opioids, nitrous, non-medication methods) **Open Module 20 →**

Dr. Oluwole's Tip

Want personalized, one-on-one guidance for your delivery? OpWell offers L&D Consultations where Dr. Oluwole reviews your complete health picture, explains every anesthesia option honestly, addresses your specific fears, and creates a plan so nothing is a surprise on delivery day. **Book your L&D Consultation →** (https://www.opwellconcierge.com/schedule)

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Created by Dr. Ornella Oluwole, Board-Certified Anesthesiologist

opwellconcierge.com (https://opwellconcierge.com) | (678) 235-5822 | dr.oluwole@opwellconcierge.com

This guide is educational and not a substitute for your physician's specific instructions. Always follow your surgeon's and anesthesiologist's personalized recommendations. Based on ASA (American Society of Anesthesiologists), ERAS (Enhanced Recovery After Surgery), and ACS (American College of Surgeons) evidence-based guidelines.